Medical Records — Bubul Das

Chronological compilation of scanned/photographed reports, 2005 – 2026 · searchable text transcription of every page, followed by the source photograph

How to read this file. Each record has a transcription page (selectable and copyable text) followed by the original photograph (rotated upright). Typed lab reports were transcribed directly; handwritten prescriptions were read by hand and are best-effort — values marked [?] are uncertain. Always check important details against the photograph. Dates are the dates printed or written on each document (DD/MM/YYYY, Indian order). This is a record compilation, not medical advice.

Index

#	Date	Document	Provider	Key content
1	2005-10-06	Prescription (handwritten)	Dr. Dhiren Kumar Nath, MBBS MS FAIMS, Consultant ENT Surgeon	Submandibular lymphadenitis with anaemia, ? hypothyroidism; Rx Dalacin C, Zerodol, Livazer-2, Smarti 10; blood/stool/RBS/urea/creatinine/platelet/PBS advised
2	2005-10-10	Pathology report — Blood R/E, RBS, Urea, Creatinine	Patholab, Dhalaibil, Sonitpur (Assam)	Hb 12.1; TC 6,400; ESR 12; RBS 60; Urea 26; Creatinine 0.9
3	2005-10-13	Pathology report — Stool R/E	Patholab, Dhalaibil, Sonitpur (Assam)	Stool: brown, semisolid; round-worm ova a few; veg cells ++
4	2005-10-20	USG Neck (7–10 MHz sector probe)	Agile Hospitals Pvt. Ltd., Radiology Section, Jayanagar Chariali, Beltola, Guwahati-781022	Small hypoechoic adenomatous nodules in both thyroid lobes; bulky right submandibular gland (reactive node 5 mm)
5	2005-10-20	Thyroid panel by chemiluminescence (T3, T4, TSH)	SRL Ranbaxy, Guwahati Lab (Dr. A. Dasgupta, MD PhD, Director–Operations)	T3 23.5 (L), T4 1.1 (L), TSH >150.00 (H) [drawn 20/10/2005]
6	2008-09-28	TSH report	Ultracare Diagnostic Centre, Beltola Survey, Guwahati-28 (Dr. Nilutpala Hazarika DCP FAGE, Consultant Pathologist; Dr. Indu Bhuyan DCP FCGP)	TSH 9.05 µIU/ml (ref 0.25–5.0)
7	2008-09-29	Prescription (handwritten)	Dr. Dhiren Kumar Nath, MBBS MS FAIMS	Thyronorm 50 mcg daily before breakfast; Mulmin cap; repeat TSH after 3 months
8	2013-03-13	Thyroid profile (T3, T4, TSH)	Thyrocare Technologies Ltd. (sample collected at Malini Lab)	T3 62; T4 3.60; TSH 1.13

9	2014-12-15	Thyroid profile (T3, T4, TSH)	Thyrocare Technologies Ltd. (sample collected at Malini Lab)	T3 53; T4 2.5; TSH 13.16 — "suggesting hypofunction"
10	2016-05-14	Thyroid profile (T3, T4, TSH)	Thyrocare Technologies Ltd. (sample collected at Malini Laboratory, Biswanath Chariali)	T3 62; T4 3.3; TSH 4.81
11	2019-08-20	Thyroid profile (T3, T4, TSH)	Thyrocare (processed at Kolkata; sample collected at Malini Laboratory, Biswanath Chariali)	T3 77; T4 3.1; TSH 14.02
12	2020-08-18	Pathology report — Random blood glucose	Malini Laboratory, NH-52, Biswanath Chariali	Blood glucose (random) 98 mg%
13	2020-08-20	Thyroid profile (T3, T4, TSH) — page 1 of 2	Thyrocare (processed at Delhi; sample collected at Malini Laboratory, Bulbul Complex, Biswanath Chariali)	T3 72; T4 4; TSH 5.71 (page 2 not photographed)
14	2023-04-10	TSH, serum (ECLIA) — pages 1 and 2	Dr. Lal PathLabs (processed at Guwahati; collected at M/S Ambika Diagnostic Centre, Tumuki near Tezpur Medical College, Tezpur)	TSH 4.98 µIU/mL (ref 0.27–4.20)
15	2024-11-25	Examination report — Rapid MP test, Typhoid IgG/IgM	Assam Clinical Laboratory, Rangachakua, Sonitpur (Assam) 784182	Rapid MP test negative; Typhoid IgG & IgM non-reactive
16	2024-11-25	Examination report — Random blood sugar	Assam Clinical Laboratory, Rangachakua, Sonitpur (Assam) 784182	Random blood sugar 110 mg/dl (up to 140)
17	2024-11-25	Prescription (handwritten)	Dr. Mridurupam Gogoi (M.D.), Medicine Specialist (certified in Diabetology), Senior Medical Officer-1,	Rx: Moxclav 625 [?], Dolo 650, Zedex M [?] etc. — largely illegible

			Kanaklata Civil Hospital, Tezpur	
18	2024-11-27	Out-patient checklist (OPD)	Dorika Hospital (a unit of Dorika LLP), Mazgaon, Tezpur	OPD consultation ticked; Dr. Piyush Agarwala MBBS MD; OPD No. 5
19	2024-11-27	OPD prescription / consultation note (handwritten)	Dorika Hospital, NH 715, Mazgaon, Tezpur-784154	Fever ~3–4 days [?]; Rx Moxclav 625 ×5 d, Dolo 650 SOS, Rivotril 0.5 [?], Zoran 1.5 gm IV ×5 d, salt gargle, Thyronorm 75 daily
20	2024-11-27	Complete blood count (CBC)	Dorika Hospital laboratory	Hb 10.5 (low); TC 14,300 (high); neutrophils 78%; MCV 74.8; platelets 1.70 lacs
21	2024-11-27	Biochemistry — Sodium, Potassium, Creatinine, CRP, TSH	Dorika Hospital laboratory	Na 128.7 (low); K 4.57; Creatinine 0.8; CRP 37.9 (high); TSH 7.86 (high)
22	2024-11-27	Urine R/E report	Dorika Hospital laboratory	Urine R/E essentially normal (SG 1.000, pH 7.0)
23	2024-11-27	X-ray chest (PA view) — report	Dorika Hospital	No active lung lesion; both CP angles blunted — pleural reaction / small effusion
24	2024-11-27	Chest X-ray film (photograph)	Dorika Hospital, NH 715, Mazgaon, Tezpur, Assam	Photo of PA chest X-ray film
25	2026-03-11	Thyroid profile (T3, T4, TSH)	Assam Laboratory ("Accurate | Caring | Instant"), NH-15, Kusumtola, Near Post Office, Sonitpur, Assam 784189	T3 0.71 (low); T4 63.45; TSH 12.63 (high)
26	2026-03-12	OPD slip with prescription (handwritten)	Rangachakua State Dispensary (ৰঙাচকুৰা ৰাজ্যিক চিকিৎসালয়), Rangachakua, Sonitpur (Assam)	Tab Thyronorm 75 mcg, ×30 [?]

Thyroid function trend (values as printed on each report)

Date	Lab	T3	T4	TSH (µIU/ml)
20/10/2005	SRL Ranbaxy	23.5 L	1.1 L	>150.00 H
28/09/2008	Ultracare	—	—	9.05
13/03/2013	Thyrocare	62	3.60	1.13
15/12/2014	Thyrocare	53	2.5	13.16
14/05/2016	Thyrocare	62	3.3	4.81
20/08/2019	Thyrocare	77	3.1	14.02
20/08/2020	Thyrocare	72	4	5.71
10/04/2023	Dr Lal PathLabs	—	—	4.98
27/11/2024	Dorika Hospital	—	—	7.86
11/03/2026	Assam Laboratory	0.71	63.45	12.63

> Units and reference ranges differ between labs (T3 ng/dl vs ng/mL; T4 µg/dl vs nmol/L) — see each report. Blank (—) = not measured in that report.

Not included: 5 photographs of other family members' reports (kept separately in the repository).

1. 06/10/2005 — Prescription (handwritten) HANDWRITTEN

Dr. Dhiren Kumar Nath, MBBS MS FAIMS, Consultant ENT Surgeon — Agile Hospital / Lokenath Medical / Goswami Clinic

Header stamp "Hosp 655…" and date **6/10/05** written at top left.

Patient: Bubul Das, 45 yrs / M

Adv (investigations advised):

- Blood R/E — with results written beside it: 6400, 69%, 28%, 02%, 01%
- Stool R/E
- RBS — 60 mg/dL
- Urea — 26
- Creatinine — 0.9
- Total platelet count
- PBS study

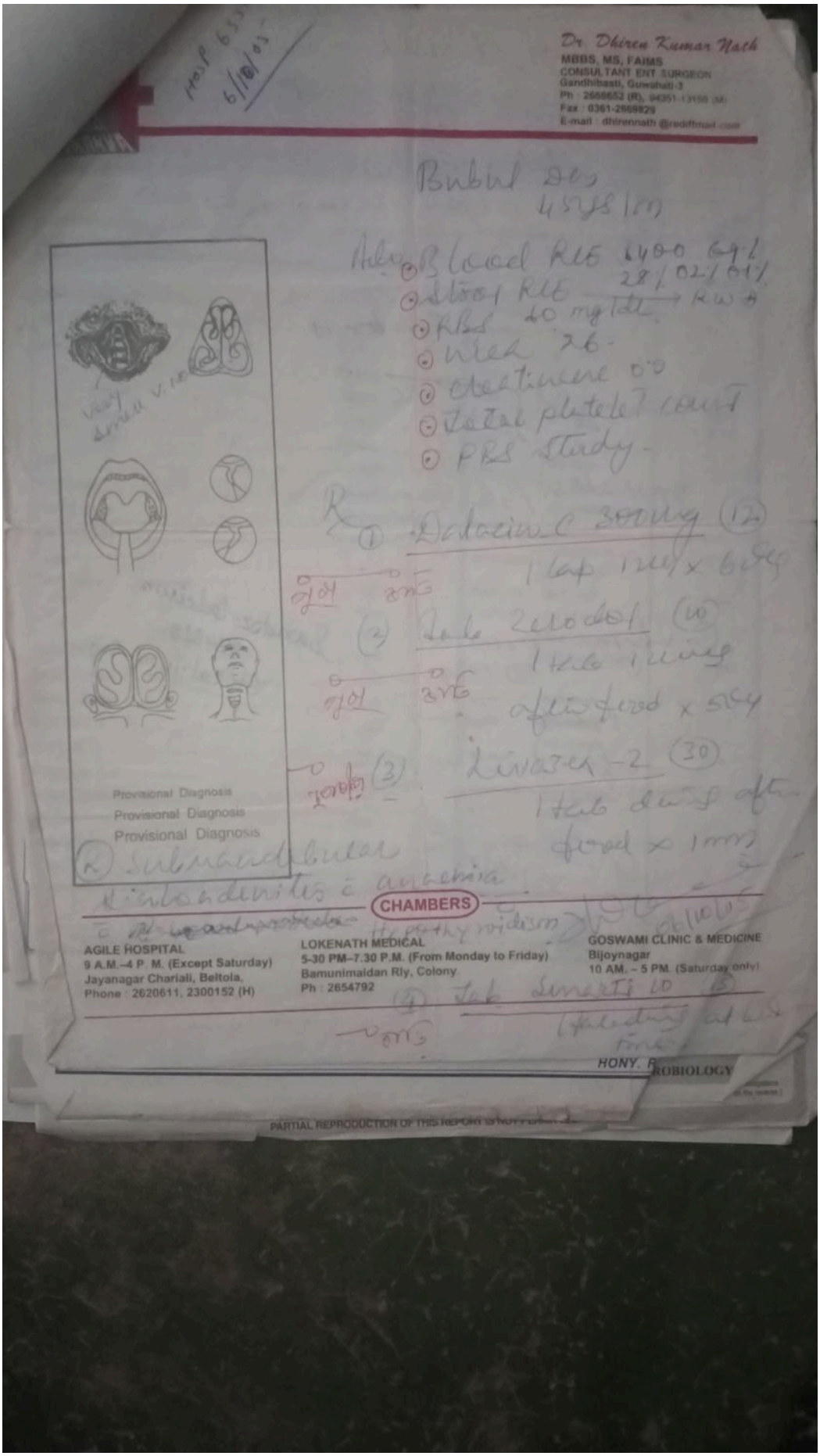
Rx:

1. Dalacin C 300 mg (12) — 1 cap 12-hrly × 6 days
2. Tab Zerodol (10) — 1 tab 12-hrly after food × 5 days
3. Livazer-2 (30) — 1 tab daily after food × 1 month
4. Tab Smarti 10 (15) — at bedtime [?]

Provisional diagnosis (written bottom left): Submandibular lymphadenitis with anaemia … [?] hypothyroidism. Signed and dated 06/10/05.

> Handwriting — best-effort transcription; items marked [?] are uncertain. Check against the image on the next page.

1. 2005-10-06 — Prescription (handwritten) · file: 2005-10-06_Prescription_Dr-DK-Nath.jpg



Dr. Dhiren Kumar Nath
MBBS, MS, FAIMS
CONSULTANT ENT SURGEON
Gandhibasti, Guwahati-3
Ph : 2668652 (R), 94351-13150 (M)
Fax : 0361-2869829
E-mail : dhirennath @rediffmail.com

Provisional Diagnosis
Provisional Diagnosis
Provisional Diagnosis

CHAMBERS

AGILE HOSPITAL
9 A.M.–4 P. M. (Except Saturday)
Jayanagar Chariali, Beltola,
Phone : 2620611, 2300152 (H)

LOKENATH MEDICAL
5-30 PM–7.30 P.M. (From Monday to Friday)
Bamunimaidan Rly. Colony
Ph : 2654792

GOSWAMI CLINIC & MEDICINE
Bijoynagar
10 AM. – 5 PM. (Saturday only)

HONY. R... ROBIOLOGY

PARTIAL REPRODUCTION OF THIS REPORT IS NOT...

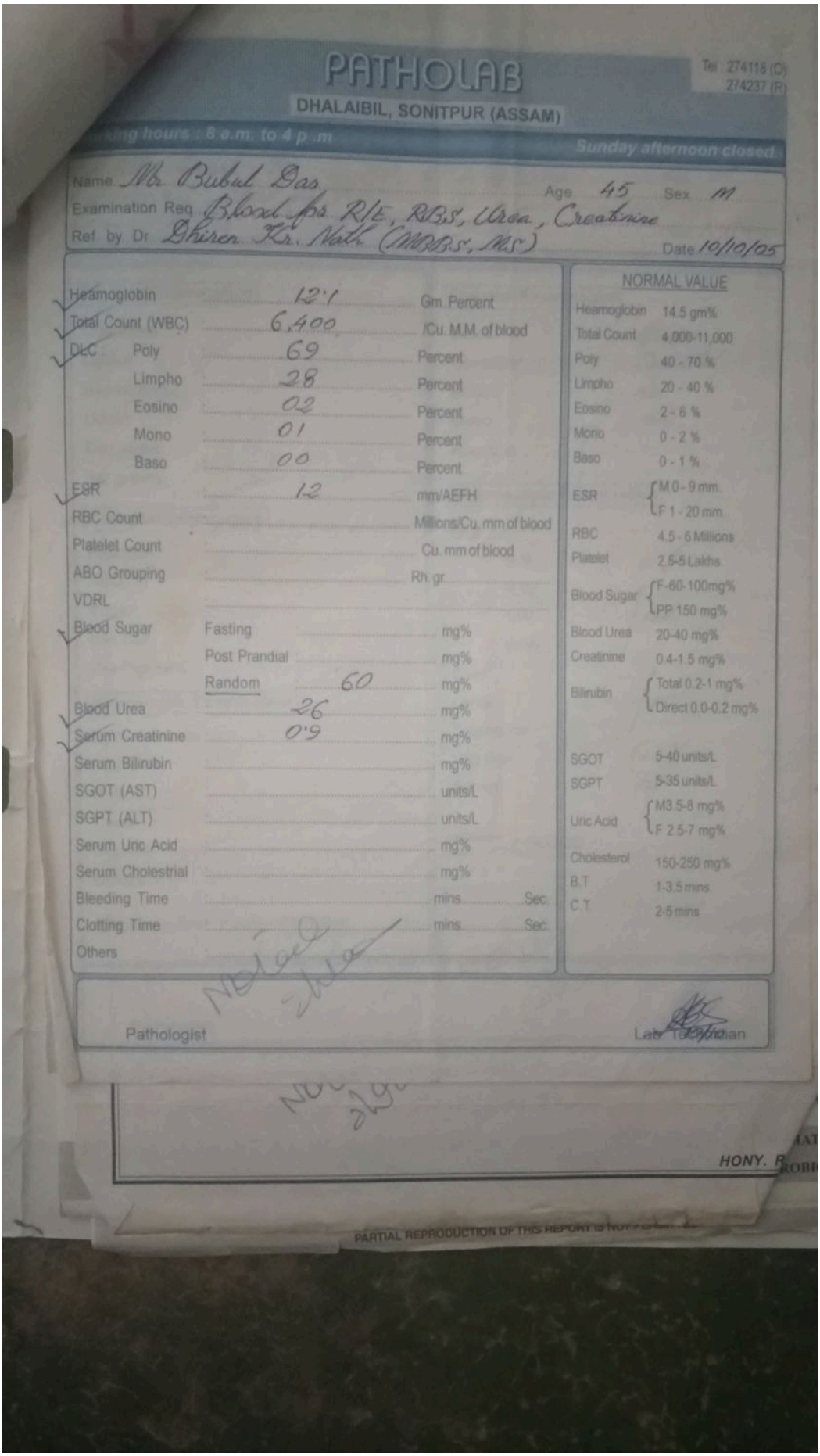
2. 10/10/2005 — Pathology report — Blood R/E, RBS, Urea, Creatinine HANDWRITTEN

Patholab, Dhalaibil, Sonitpur (Assam)

Patient: Mr. Bubul Das · Age 45 · Sex M · **Date:** 10/10/05
Examination requested: Blood for R/E, RBS, Urea, Creatinine · **Ref. by:** Dhiren Kr. Nath (MBBS, MS)

Test	Result	Unit	Normal value
Haemoglobin	12.1	Gm. Percent	14.5 gm%
Total Count (WBC)	6,400	/Cu.mm of blood	4,000–11,000
DLC — Poly	69	Percent	40–70 %
Lympho	28	Percent	20–40 %
Eosino	02	Percent	2–6 %
Mono	01	Percent	0–2 %
Baso	00	Percent	0–1 %
ESR	12	mm/AEFH	M 0–9 mm; F 1–20 mm
Blood Sugar — Random	60	mg%	F 60–100 mg%; PP 150 mg%
Blood Urea	26	mg%	20–40 mg%
Serum Creatinine	0.9	mg%	0.4–1.5 mg%

2. 2005-10-10 — Pathology report — Blood R/E, RBS, Urea, Creatinine · file: 2005-10-10_Blood-Report_Patholab.jpg



PATHOLAB

Tel : 274118 (O)
274237 (R)

DHALAIBIL, SONITPUR (ASSAM)

Working hours : 8 a.m. to 4 p.m. Sunday afternoon closed.

Name Mr. Bubul Das. Age 45 Sex M

Examination Req. Blood for R/E, RBS, Urea, Creatinine

Ref. by Dr. Dhiren Kr. Nath (MBBS, MS) Date 10/10/05

Heamoglobin	12·1	Gm. Percent	
Total Count (WBC)	6,400	/Cu. M.M. of blood	
DLC Poly	69	Percent	
Limpho	28	Percent	
Eosino	02	Percent	
Mono	01	Percent	
Baso	00	Percent	
ESR	12	mm/AEFH	
RBC Count		Millions/Cu. mm of blood	
Platelet Count		Cu. mm of blood	
ABO Grouping		Rh. gr.	
VDRL			
Blood Sugar Fasting		mg%	
Post Prandial		mg%	
Random	60	mg%	
Blood Urea	26	mg%	
Serum Creatinine	0·9	mg%	
Serum Bilirubin		mg%	
SGOT (AST)		units/L	
SGPT (ALT)		units/L	
Serum Uric Acid		mg%	
Serum Cholestrial		mg%	
Bleeding Time		mins. Sec.	
Clotting Time		mins. Sec.	
Others			

NORMAL VALUE

Heamoglobin	14.5 gm%
Total Count	4,000-11,000
Poly	40 - 70 %
Limpho	20 - 40 %
Eosino	2 - 6 %
Mono	0 - 2 %
Baso	0 - 1 %
ESR	M 0 - 9 mm. F 1 - 20 mm.
RBC	4.5 - 6 Millions
Platelet	2.5-5 Lakhs
Blood Sugar	F-60-100mg% PP-150 mg%
Blood Urea	20-40 mg%
Creatinine	0.4-1.5 mg%
Bilirubin	Total 0.2-1 mg% Direct 0.0-0.2 mg%
SGOT	5-40 units/L
SGPT	5-35 units/L
Uric Acid	M 3.5-8 mg% F 2.5-7 mg%
Cholesterol	150-250 mg%
B.T	1-3.5 mins
C.T	2-5 mins

Pathologist

Lab Technician

HONY. R

PARTIAL REPRODUCTION OF THIS REPORT IS NOT

3. 13/10/2005 — Pathology report — Stool R/E HANDWRITTEN

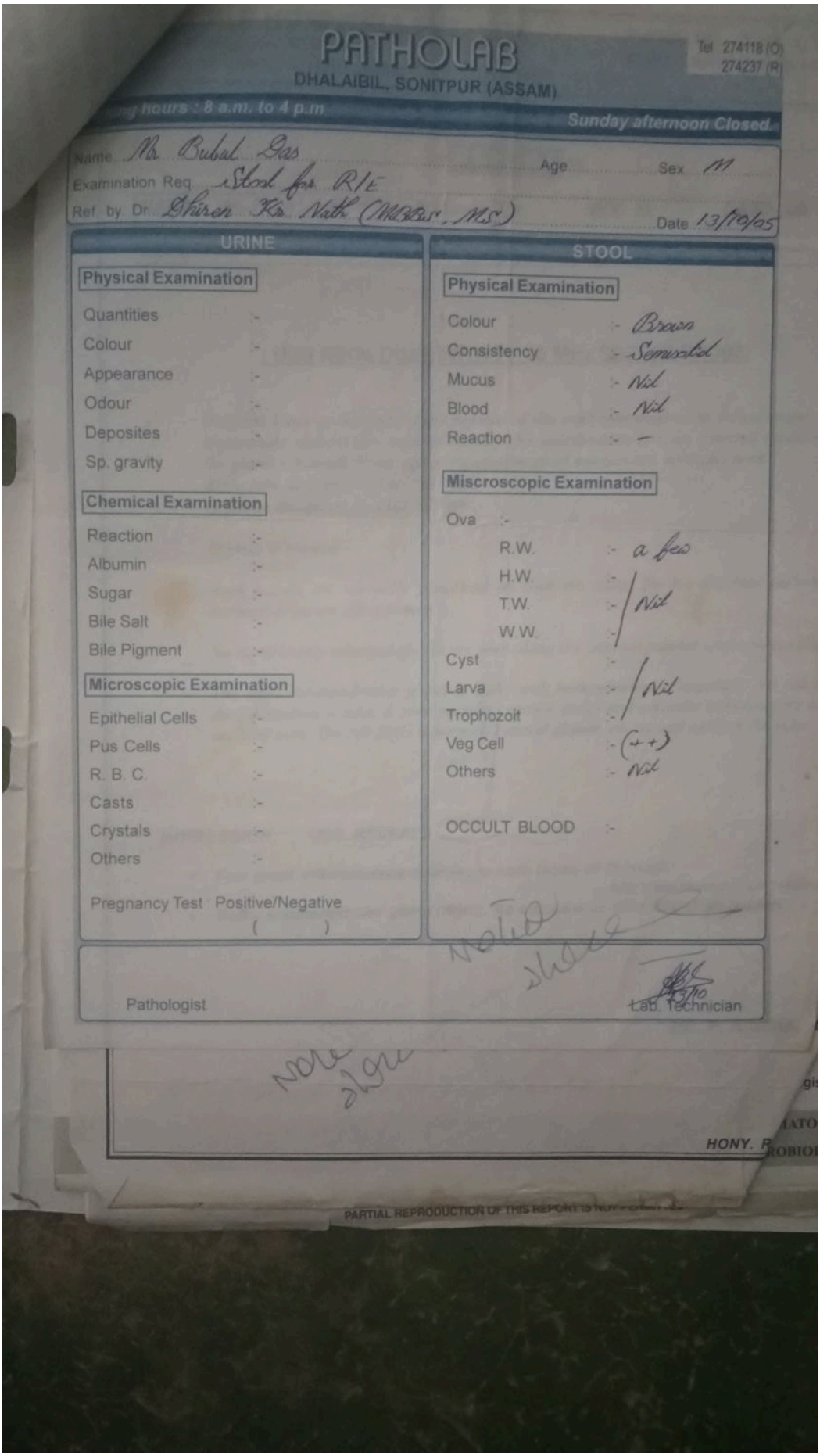
Patholab, Dhalaibil, Sonitpur (Assam)

Patient: Mr. Bubul Das · Sex M · **Date:** 13/10/05
Examination requested: Stool for R/E · **Ref. by:** Dhiren Kr. Nath (MBBS, MS)

Stool — Physical / Microscopic	Result
Colour	Brown
Consistency	Semisolid
Mucus	Nil
Blood	Nil
Reaction	—
Ova — R.W. (round worm)	a few
Ova — H.W. / T.W. / W.W.	Nil
Cyst	Nil
Larva	Nil
Trophozoite	(blank)
Veg Cell	(++)
Others	Nil
Occult blood	(blank)

The Urine section of the form is blank. Handwritten values.

3. 2005-10-13 — Pathology report — Stool R/E · file: 2005-10-13_Stool-Report_Patholab.jpg



PATHOLAB

DHALAIBIL, SONITPUR (ASSAM)

Tel 274118 (O)
274237 (R)

hours : 8 a.m. to 4 p.m — Sunday afternoon Closed

Name Mr. Bubal Das Age Sex M

Examination Req. Stool for R/E

Ref. by Dr. Ghiren Kr. Nath (MBBS, MS) Date 13/10/05

URINE

Physical Examination

Quantities :-
Colour :-
Appearance :-
Odour :-
Deposites :-
Sp. gravity :-

Chemical Examination

Reaction :-
Albumin :-
Sugar :-
Bile Salt :-
Bile Pigment :-

Microscopic Examination

Epithelial Cells :-
Pus Cells :-
R. B. C. :-
Casts :-
Crystals :-
Others :-

Pregnancy Test Positive/Negative
( )

STOOL

Physical Examination

Colour :- Brown
Consistency :- Semisolid
Mucus :- Nil
Blood :- Nil
Reaction :- –

Miscroscopic Examination

Ova :-
R.W. :- a few
H.W. :-
T.W. :- Nil
W.W. :-
Cyst :-
Larva :- Nil
Trophozoit :-
Veg Cell :- (++)
Others :- Nil

OCCULT BLOOD :-

Pathologist

Lab. Technician

HONY.

PARTIAL REPRODUCTION OF THIS REPORT IS

4. 20/10/2005 — USG Neck (7–10 MHz sector probe)

Agile Hospitals Pvt. Ltd., Radiology Section, Jayanagar Chariali, Beltola, Guwahati-781022

Name: Ms. [sic] Babul Das · **Age:** 45 Yrs · **Sex:** M · **Date:** 20.10.05 · **No:** 67
Referred by: Dr. D.K. Nath, MS.

USG NECK DONE WITH 7–10 MHz SECTOR PROBE:

Both the lobes of the thyroid are normal in size with inhomogeneous echoanatomy. Small hypoechoic nodules are seen bilaterally. No calcification is seen. Internal vascularity of the gland is normal. Strap and sternocleidomastoid muscles are normally seen.
Right lobe measures = 14×17×37 mm.
Left lobe measures = 15×17×29 mm.

Isthmus is normal.

Neck vessels are normally visualized on both the sides. No intraluminal pathology or extrinsic pressure effect is seen.

No significantly enlarged glands are seen along the internal jugular chain bilaterally.

The right Submandibular gland is bulky with homogeneous echotexture. No calculus or duct dilatation is seen. A 5 mm nodule (reactive node) and adjacent inflammatory changes are also seen. The left SMG is normal. Parotid glands are normal on both the sides.

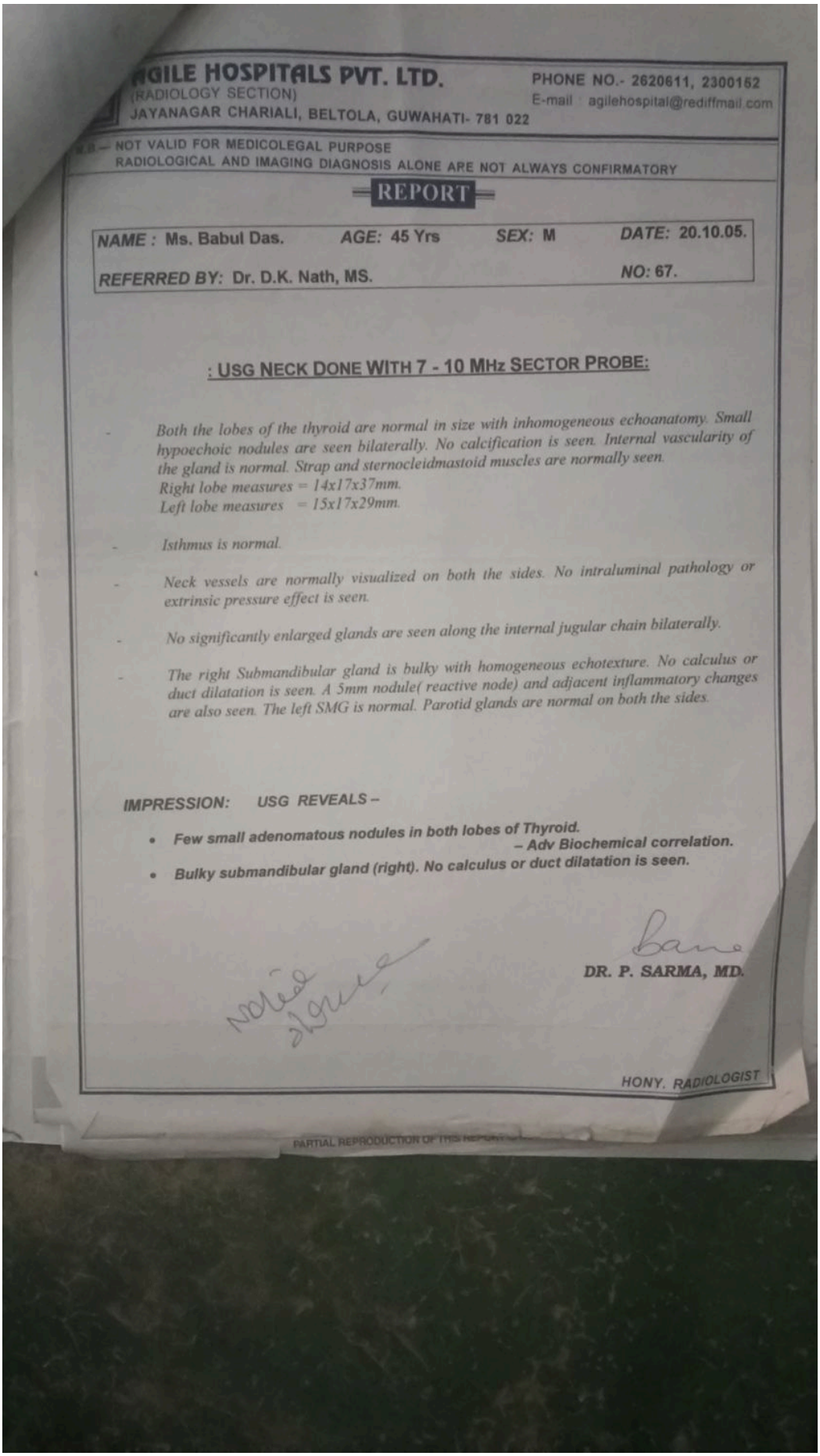
IMPRESSION — USG REVEALS:

- Few small adenomatous nodules in both lobes of Thyroid. — Adv Biochemical correlation.
- Bulky submandibular gland (right). No calculus or duct dilatation is seen.

Dr. P. Sarma, MD, Hony. Radiologist.

> Report header prints the NOT VALID FOR MEDICOLEGAL PURPOSE disclaimer: radiological and imaging diagnosis alone are not always confirmatory.

4. 2005-10-20 — USG Neck (7–10 MHz sector probe) · file: 2005-10-20_USG-Neck_Agile-Hospitals.jpg



AGILE HOSPITALS PVT. LTD.
(RADIOLOGY SECTION)
JAYANAGAR CHARIALI, BELTOLA, GUWAHATI- 781 022
PHONE NO.- 2620611, 2300152
E-mail : agilehospital@rediffmail.com

N.B.— NOT VALID FOR MEDICOLEGAL PURPOSE
RADIOLOGICAL AND IMAGING DIAGNOSIS ALONE ARE NOT ALWAYS CONFIRMATORY

REPORT

NAME : Ms. Babul Das. AGE: 45 Yrs SEX: M DATE: 20.10.05.
REFERRED BY: Dr. D.K. Nath, MS. NO: 67.

: USG NECK DONE WITH 7 - 10 MHz SECTOR PROBE:

- Both the lobes of the thyroid are normal in size with inhomogeneous echoanatomy. Small hypoechoic nodules are seen bilaterally. No calcification is seen. Internal vascularity of the gland is normal. Strap and sternocleidmastoid muscles are normally seen.
  Right lobe measures = 14x17x37mm.
  Left lobe measures = 15x17x29mm.
- Isthmus is normal.
- Neck vessels are normally visualized on both the sides. No intraluminal pathology or extrinsic pressure effect is seen.
- No significantly enlarged glands are seen along the internal jugular chain bilaterally.
- The right Submandibular gland is bulky with homogeneous echotexture. No calculus or duct dilatation is seen. A 5mm nodule(reactive node) and adjacent inflammatory changes are also seen. The left SMG is normal. Parotid glands are normal on both the sides.

IMPRESSION: USG REVEALS –

- Few small adenomatous nodules in both lobes of Thyroid.
  – Adv Biochemical correlation.
- Bulky submandibular gland (right). No calculus or duct dilatation is seen.

DR. P. SARMA, MD.
HONY. RADIOLOGIST

PARTIAL REPRODUCTION OF THIS REPORT

5. 20/10/2005 — Thyroid panel by chemiluminescence (T3, T4, TSH)

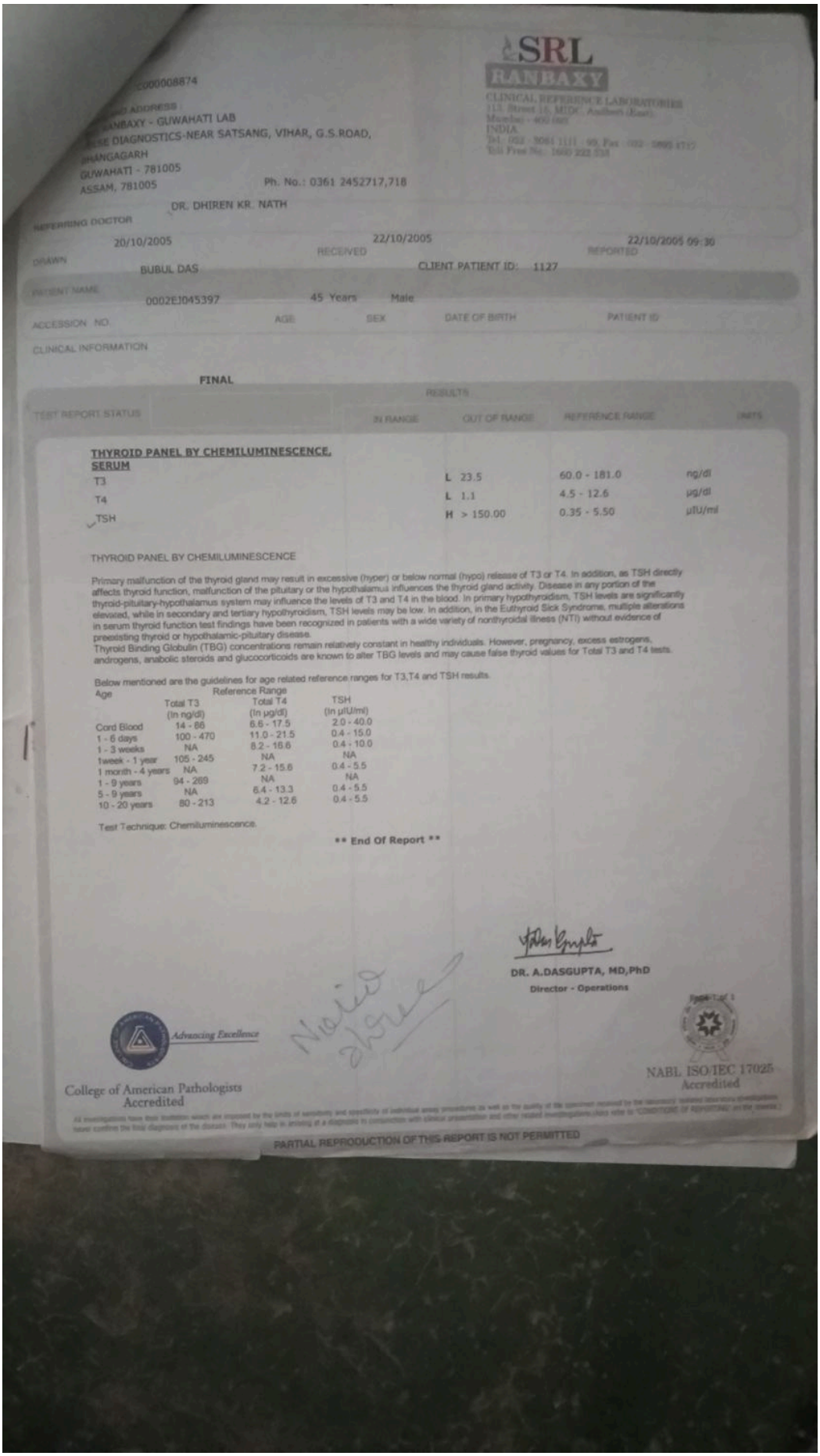
SRL Ranbaxy, Guwahati Lab (Dr. A. Dasgupta, MD PhD, Director–Operations)

Patient: Bubul Das · 45 Years · Male · **Referring doctor:** Dr. Dhiren Kr. Nath
Drawn: 20/10/2005 · **Received:** 22/10/2005 · **Reported:** 22/10/2005 09:30 · Accession 0002E1045397 · Client patient ID 1127 · Status FINAL

Test	Flag	Result	Reference range	Units
T3	L	23.5	60.0 – 181.0	ng/dl
T4	L	1.1	4.5 – 12.6	µg/dl
TSH	H	> 150.00	0.35 – 5.50	µIU/ml

Comments printed on report: primary malfunction of the thyroid gland may result in excessive (hyper) or below normal (hypo) release of T3 or T4. TSH is secreted by the adenohypophysis and is controlled by negative feedback from circulating T3, T4 and TRH; in primary hypothyroidism TSH levels are elevated, in secondary and tertiary hypothyroidism they may be low. Age-related reference ranges are tabulated on the report (e.g. adults 20+ yrs: T4 4.2–12.6 µg/dl). Test technique: Chemiluminescence. NABL ISO/IEC 17025 and CAP accredited.

5. 2005-10-20 — Thyroid panel by chemiluminescence (T3, T4, TSH) · file: 2005-10-20_Thyroid-Panel_SRL-Ranbaxy.jpg



SRL
RANBAXY
CLINICAL REFERENCE LABORATORIES
115, Street 16, MIDC, Andheri (East),
Mumbai - 400 093
INDIA
Tel.: 022 - 3061 1111 - 99, Fax : 022 - 3093 4737
Toll Free No.: 1600 222 531

C000008874

ADDRESS
RANBAXY - GUWAHATI LAB
BASE DIAGNOSTICS-NEAR SATSANG, VIHAR, G.S.ROAD,
DHANGAGARH
GUWAHATI - 781005
ASSAM, 781005
Ph. No.: 0361 2452717,718

REFERRING DOCTOR: DR. DHIREN KR. NATH

DRAWN	RECEIVED	REPORTED
20/10/2005	22/10/2005	22/10/2005 09:30

PATIENT NAME: BUBUL DAS — CLIENT PATIENT ID: 1127

ACCESSION NO.	AGE	SEX	DATE OF BIRTH	PATIENT ID
0002EJ045397	45 Years	Male		

CLINICAL INFORMATION

FINAL

TEST REPORT STATUS	RESULTS: IN RANGE	OUT OF RANGE	REFERENCE RANGE	UNITS
THYROID PANEL BY CHEMILUMINESCENCE, SERUM				
T3		L 23.5	60.0 - 181.0	ng/dl
T4		L 1.1	4.5 - 12.6	µg/dl
TSH		H > 150.00	0.35 - 5.50	µIU/ml

THYROID PANEL BY CHEMILUMINESCENCE

Primary malfunction of the thyroid gland may result in excessive (hyper) or below normal (hypo) release of T3 or T4. In addition, as TSH directly affects thyroid function, malfunction of the pituitary or the hypothalamus influences the thyroid gland activity. Disease in any portion of the thyroid-pituitary-hypothalamus system may influence the levels of T3 and T4 in the blood. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels may be low. In addition, in the Euthyroid Sick Syndrome, multiple alterations in serum thyroid function test findings have been recognized in patients with a wide variety of nonthyroidal illness (NTI) without evidence of preexisting thyroid or hypothalamic-pituitary disease.
Thyroid Binding Globulin (TBG) concentrations remain relatively constant in healthy individuals. However, pregnancy, excess estrogens, androgens, anabolic steroids and glucocorticoids are known to alter TBG levels and may cause false thyroid values for Total T3 and T4 tests.

Below mentioned are the guidelines for age related reference ranges for T3,T4 and TSH results.

Age	Reference Range Total T3 (In ng/dl)	Total T4 (In µg/dl)	TSH (In µIU/ml)
Cord Blood	14 - 86	6.6 - 17.5	2.0 - 40.0
1 - 6 days	100 - 470	11.0 - 21.5	0.4 - 15.0
1 - 3 weeks	NA	8.2 - 18.6	0.4 - 10.0
1week - 1 year	105 - 245	NA	NA
1 month - 4 years	NA	7.2 - 15.6	0.4 - 5.5
1 - 9 years	94 - 269	NA	NA
5 - 9 years	NA	6.4 - 13.3	0.4 - 5.5
10 - 20 years	80 - 213	4.2 - 12.6	0.4 - 5.5

Test Technique: Chemiluminescence.

** End Of Report **

DR. A.DASGUPTA, MD,PhD
Director - Operations

Advancing Excellence
College of American Pathologists
Accredited

NABL ISO/IEC 17025
Accredited

All investigations have their limitation which are imposed by the limits of sensitivity and specificity of individual assay procedures as well as the quality of the specimen received by the laboratory. Laboratory investigations have confirm the final diagnosis of the disease. They only help in arriving at a diagnosis in conjunction with clinical presentation and other related investigations. Also refer to "CONDITIONS OF REPORTING" on the reverse.

PARTIAL REPRODUCTION OF THIS REPORT IS NOT PERMITTED

6. 28/09/2008 — TSH report

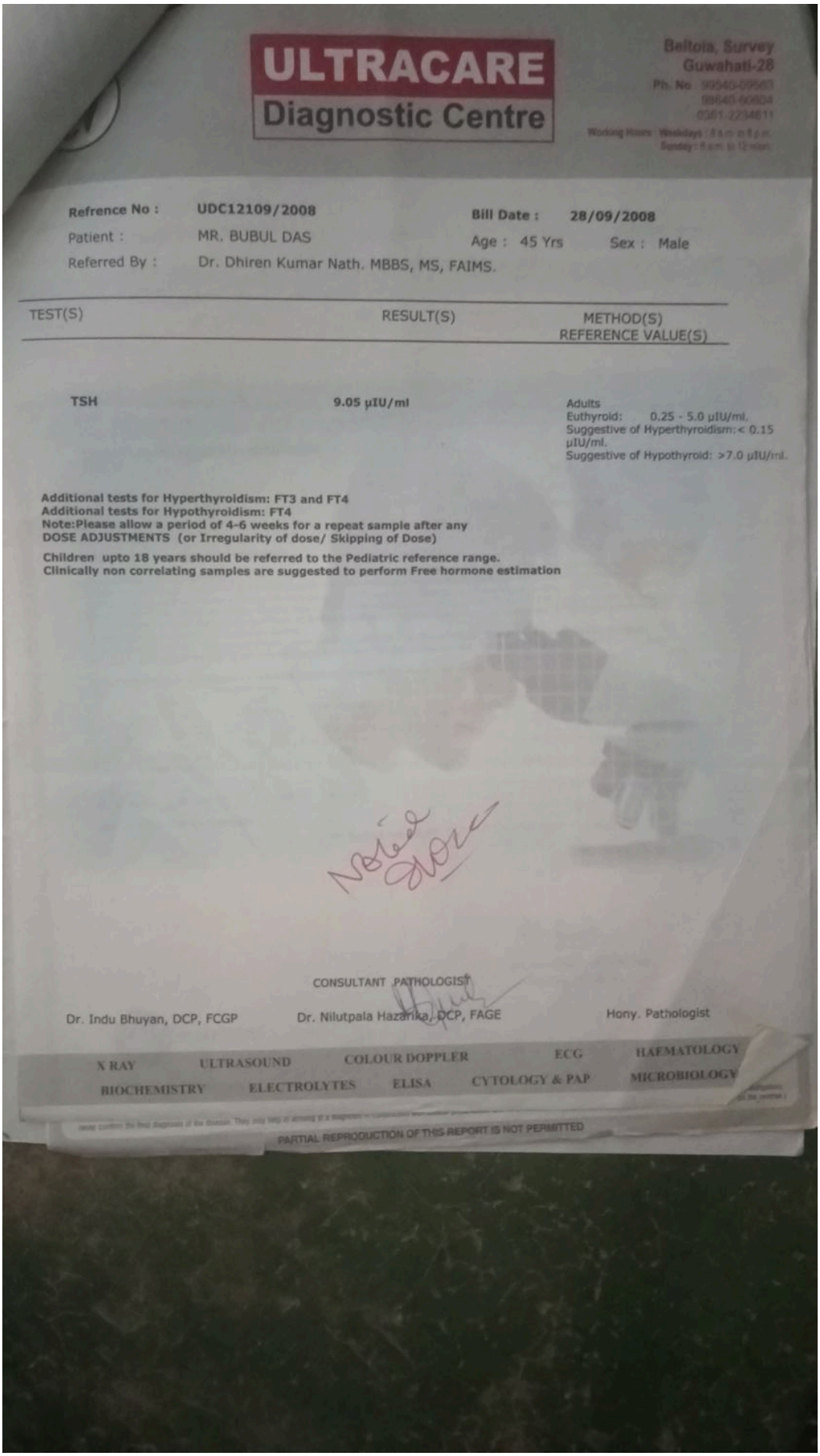
Ultracare Diagnostic Centre, Beltola Survey, Guwahati-28 (Dr. Nilutpala Hazarika DCP FAGE, Consultant Pathologist; Dr. Indu Bhuyan DCP FCGP)

Reference No: UDC12109/2008 · **Bill date:** 28/09/2008
Patient: Mr. Bubul Das · **Age:** 45 Yrs · **Sex:** Male
Referred by: Dr. Dhiren Kumar Nath, MBBS, MS, FAIMS

Test	Result	Reference value
TSH	9.05 µIU/ml	Adults — Euthyroid: 0.25–5.0 µIU/ml; suggestive of hyperthyroidism: < 0.15 µIU/ml; suggestive of hypothyroid: > 7.0 µIU/ml

Notes printed: Additional tests for hyperthyroidism: FT3 and FT4; for hypothyroidism: FT4. Please allow a period of 4–6 weeks for a repeat sample after any dose adjustments (or irregularity of dose / skipping of dose). Children up to 18 years should be referred to the Paediatric reference range. Clinically non-correlating samples are suggested to perform free hormone estimation.

6. 2008-09-28 — TSH report · file: 2008-09-28_TSH_Ultracare.jpg



ULTRACARE
Diagnostic Centre

Beltola, Survey
Guwahati-28
Ph. No : 93640-09563
98640-60604
0361-2234611
Working Hours : Weekdays : 8 a.m. to 8 p.m.
Sunday : 8 a.m. to 12 noon

Refrence No : UDC12109/2008 **Bill Date :** 28/09/2008

Patient : MR. BUBUL DAS Age : 45 Yrs Sex : Male

Referred By : Dr. Dhiren Kumar Nath. MBBS, MS, FAIMS.

TEST(S)	RESULT(S)	METHOD(S) REFERENCE VALUE(S)
TSH	**9.05 µIU/ml**	Adults Euthyroid: 0.25 - 5.0 µIU/ml. Suggestive of Hyperthyroidism: < 0.15 µIU/ml. Suggestive of Hypothyroid: >7.0 µIU/ml.

Additional tests for Hyperthyroidism: FT3 and FT4
Additional tests for Hypothyroidism: FT4
Note:Please allow a period of 4-6 weeks for a repeat sample after any DOSE ADJUSTMENTS (or Irregularity of dose/ Skipping of Dose)

Children upto 18 years should be referred to the Pediatric reference range.
Clinically non correlating samples are suggested to perform Free hormone estimation

CONSULTANT PATHOLOGIST

Dr. Indu Bhuyan, DCP, FCGP Dr. Nilutpala Hazarika, DCP, FAGE Hony. Pathologist

X RAY ULTRASOUND COLOUR DOPPLER ECG HAEMATOLOGY
BIOCHEMISTRY ELECTROLYTES ELISA CYTOLOGY & PAP MICROBIOLOGY

PARTIAL REPRODUCTION OF THIS REPORT IS NOT PERMITTED

7. 29/09/2008 — Prescription (handwritten) HANDWRITTEN

Dr. Dhiren Kumar Nath, MBBS MS FAIMS — Agile Hospital / Lokenath Medical

Patient: Mr. Bubul Das, 45 yrs / M · **Date:** 29.09.08

BP 120/80 mmHg
Chest — clear
TSH on 28/09/08 — 9.05 µIU/mL

Rx:

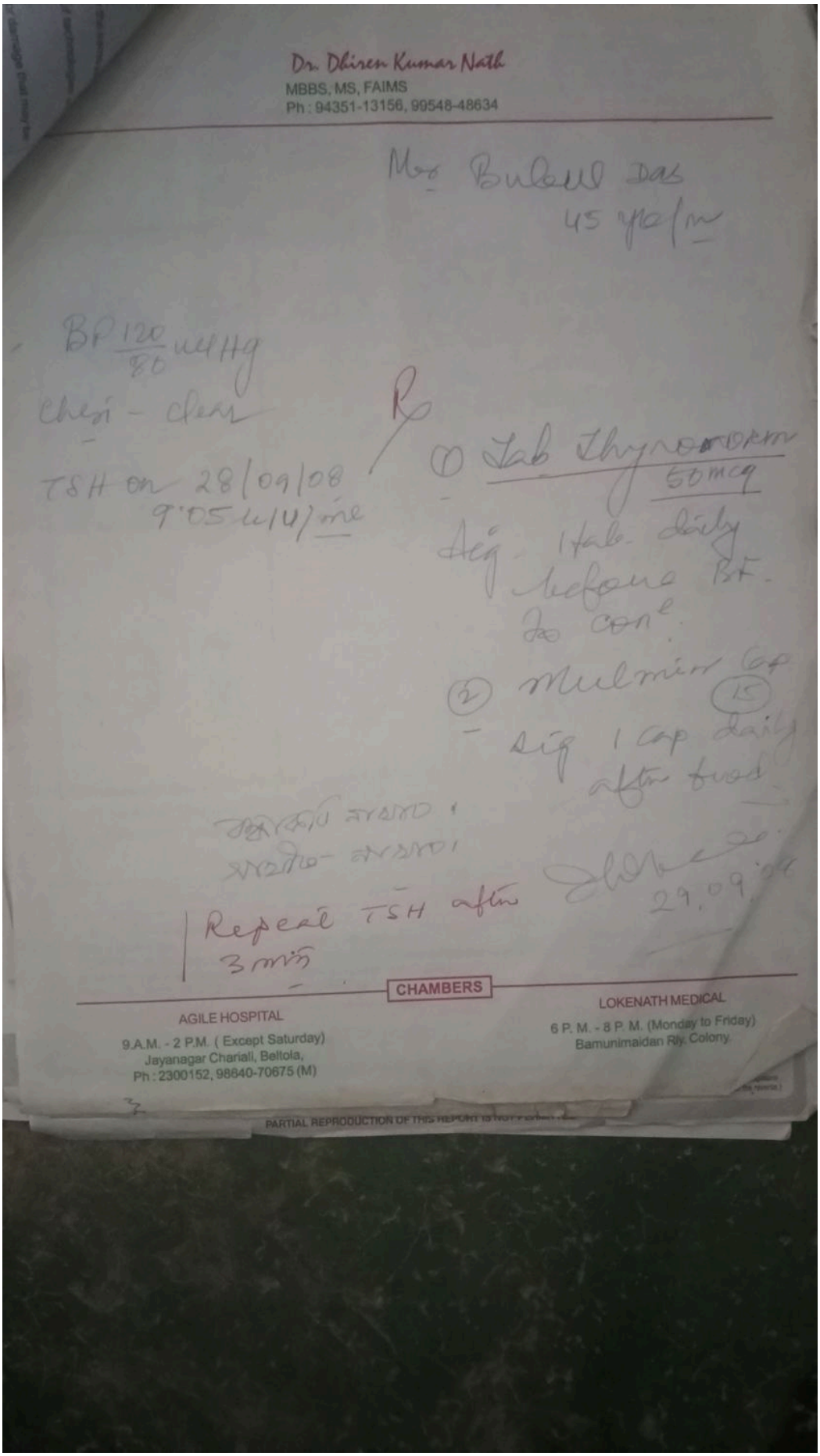
1. Tab Thyronorm 50 mcg — Sig: 1 tab daily before breakfast. To continue.
2. Mulmin cap (15) — Sig: 1 cap daily after food.

(Two lines of Bengali/Assamese script — not transcribed.)

Repeat TSH after 3 months.

> Handwriting — best-effort transcription.

7. 2008-09-29 — Prescription (handwritten) · file: 2008-09-29_Prescription_Dr-DK-Nath.jpg



Dr. Dhiren Kumar Nath
MBBS, MS, FAIMS
Ph : 94351-13156, 99548-48634

Mr. Bulbul Das
45 yrs/M

BP 120/80 mmHg
Chest - clear

TSH on 28/09/08
9.05 mIU/ml

℞
① Tab Thyronorm 50mcg
Sig - 1 tab. daily
before BF.
for one month.

② Mulmin Cap (15)
- Sig 1 cap daily
after food

Repeat TSH after
3 months

29.09.08

CHAMBERS

AGILE HOSPITAL
9.A.M. - 2 P.M. (Except Saturday)
Jayanagar Chariali, Beltola,
Ph : 2300152, 98640-70675 (M)

LOKENATH MEDICAL
6 P. M. - 8 P. M. (Monday to Friday)
Bamunimaidan Rly. Colony

PARTIAL REPRODUCTION OF THIS REPORT IS NOT

8. 13/03/2013 — Thyroid profile (T3, T4, TSH)

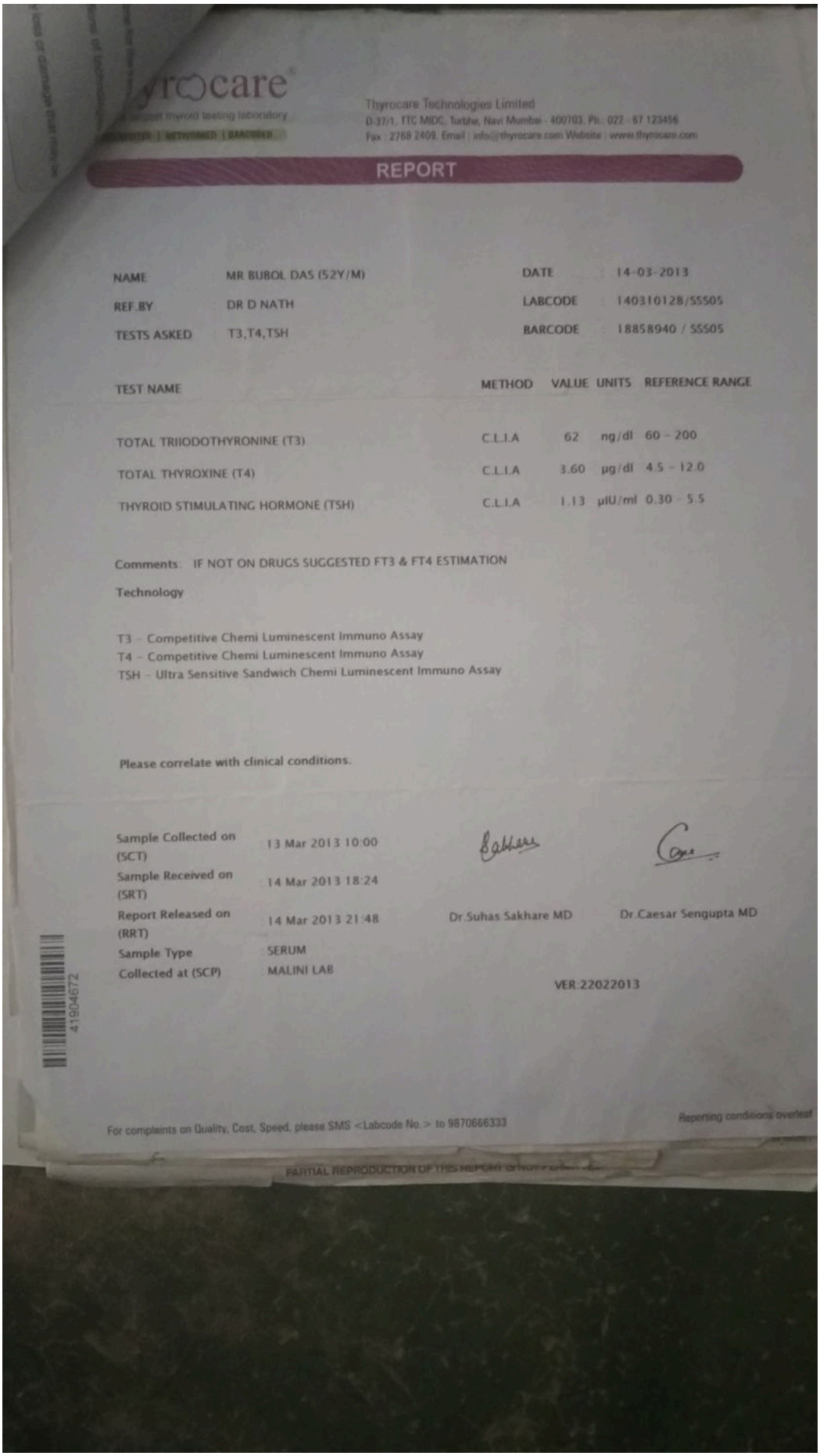
Thyrocare Technologies Ltd. (sample collected at Malini Lab)

Name: Mr Bubol Das (52Y/M) · **Ref. by:** Dr D Nath · **Tests:** T3, T4, TSH
Report date: 14-03-2013 · Labcode 140310128/SS505 · Barcode 18858940/SS505
Collected (SCT): 13 Mar 2013 10:00 · **Received:** 14 Mar 2013 18:24 · **Released:** 14 Mar 2013 21:48 · Serum · Collected at: Malini Lab

Test	Method	Value	Units	Reference range
Total Triiodothyronine (T3)	C.L.I.A	62	ng/dl	60–200
Total Thyroxine (T4)	C.L.I.A	3.60	µg/dl	4.5–12.0
Thyroid Stimulating Hormone (TSH)	C.L.I.A	1.13	µIU/ml	0.30–5.5

Comments: If not on drugs suggested FT3 & FT4 estimation. Please correlate with clinical conditions.
Signed: Dr. Suhas Sakhare MD; Dr. Caesar Sengupta MD.

8. 2013-03-13 — Thyroid profile (T3, T4, TSH) · file: 2013-03-13_Thyroid-Profile_Thyrocare.jpg



yrocare®
...test thyroid testing laboratory
...| NETWORKED | BARCODED

Thyrocare Technologies Limited
D-37/1, TTC MIDC, Turbhe, Navi Mumbai - 400703. Ph.: 022 - 67 123456
Fax : 2768 2409. Email : info@thyrocare.com Website : www.thyrocare.com

REPORT

NAME	MR BUBOL DAS (52Y/M)	DATE	14-03-2013
REF.BY	DR D NATH	LABCODE	140310128/SSS05
TESTS ASKED	T3,T4,TSH	BARCODE	18858940 / SSS05

TEST NAME	METHOD	VALUE	UNITS	REFERENCE RANGE
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	62	ng/dl	60 - 200
TOTAL THYROXINE (T4)	C.L.I.A	3.60	µg/dl	4.5 - 12.0
THYROID STIMULATING HORMONE (TSH)	C.L.I.A	1.13	µIU/ml	0.30 - 5.5

Comments: IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

Technology

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - Ultra Sensitive Sandwich Chemi Luminescent Immuno Assay

Please correlate with clinical conditions.

Sample Collected on (SCT)	13 Mar 2013 10:00
Sample Received on (SRT)	14 Mar 2013 18:24
Report Released on (RRT)	14 Mar 2013 21:48
Sample Type	SERUM
Collected at (SCP)	MALINI LAB

Dr.Suhas Sakhare MD Dr.Caesar Sengupta MD

VER:22022013

41904672

For complaints on Quality, Cost, Speed, please SMS <Labcode No.> to 9870666333

Reporting conditions overleaf

PARTIAL REPRODUCTION OF THIS REPORT IS NOT P...

9. 15/12/2014 — Thyroid profile (T3, T4, TSH)

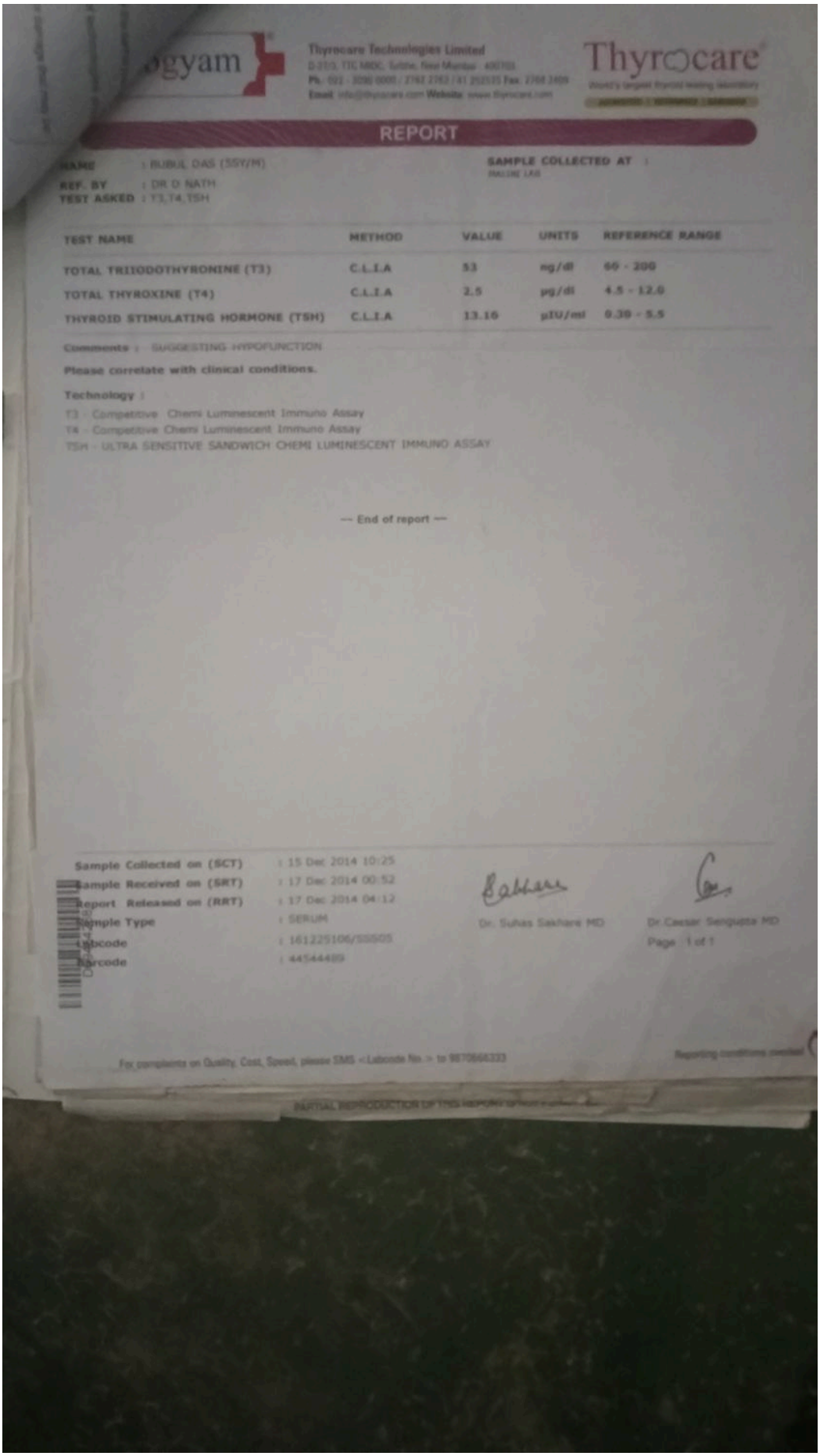
Thyrocare Technologies Ltd. (sample collected at Malini Lab)

Name: Bubul Das (55Y/M) · **Ref. by:** Dr D Nath · **Tests:** T3, T4, TSH
Collected (SCT): 15 Dec 2014 10:25 · **Received:** 17 Dec 2014 00:52 · **Released:** 17 Dec 2014 04:12 · Serum · Labcode 161225106/SS505 · Barcode 44544489

Test	Method	Value	Units	Reference range
Total Triiodothyronine (T3)	C.L.I.A	53	ng/dl	60–200
Total Thyroxine (T4)	C.L.I.A	2.5	µg/dl	4.5–12.0
Thyroid Stimulating Hormone (TSH)	C.L.I.A	13.16	µIU/ml	0.30–5.5

Comments: SUGGESTING HYPOFUNCTION. Please correlate with clinical conditions. Signed: Dr. Suhas Sakhare MD; Dr. Caesar Sengupta MD.

9. 2014-12-15 — Thyroid profile (T3, T4, TSH) · file: 2014-12-15_Thyroid-Profile_Thyrocare.jpg



ogyam

Thyrocare Technologies Limited
D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400703
Ph.: 022 - 3090 0000 / 2762 2762 / 61 252525 Fax: 2768 2409
Email: info@thyrocare.com Website: www.thyrocare.com

Thyrocare
World's largest thyroid testing laboratory

REPORT

NAME : BUBUL DAS (55Y/M)
REF. BY : DR D NATH
TEST ASKED : T3,T4,TSH

SAMPLE COLLECTED AT :
IMAGINE LAB

TEST NAME	METHOD	VALUE	UNITS	REFERENCE RANGE
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	53	ng/dl	60 - 200
TOTAL THYROXINE (T4)	C.L.I.A	2.5	µg/dl	4.5 - 12.0
THYROID STIMULATING HORMONE (TSH)	C.L.I.A	13.16	µIU/ml	0.38 - 5.5

Comments : SUGGESTING HYPOFUNCTION

Please correlate with clinical conditions.

Technology :
T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - ULTRA SENSITIVE SANDWICH CHEMI LUMINESCENT IMMUNO ASSAY

--- End of report ---

Sample Collected on (SCT) : 15 Dec 2014 10:25
Sample Received on (SRT) : 17 Dec 2014 00:52
Report Released on (RRT) : 17 Dec 2014 04:12
Sample Type : SERUM
Labcode : 161225106/SS505
Barcode : 44544409

Dr. Suhas Sakhare MD

Dr.Caesar Sengupta MD

Page 1 of 1

For complaints on Quality, Cost, Speed, please SMS <Labcode No.> to 9870866333

Reporting conditions overleaf

PARTIAL REPRODUCTION OF THIS REPORT IS NOT VALID

10. 14/05/2016 — Thyroid profile (T3, T4, TSH)

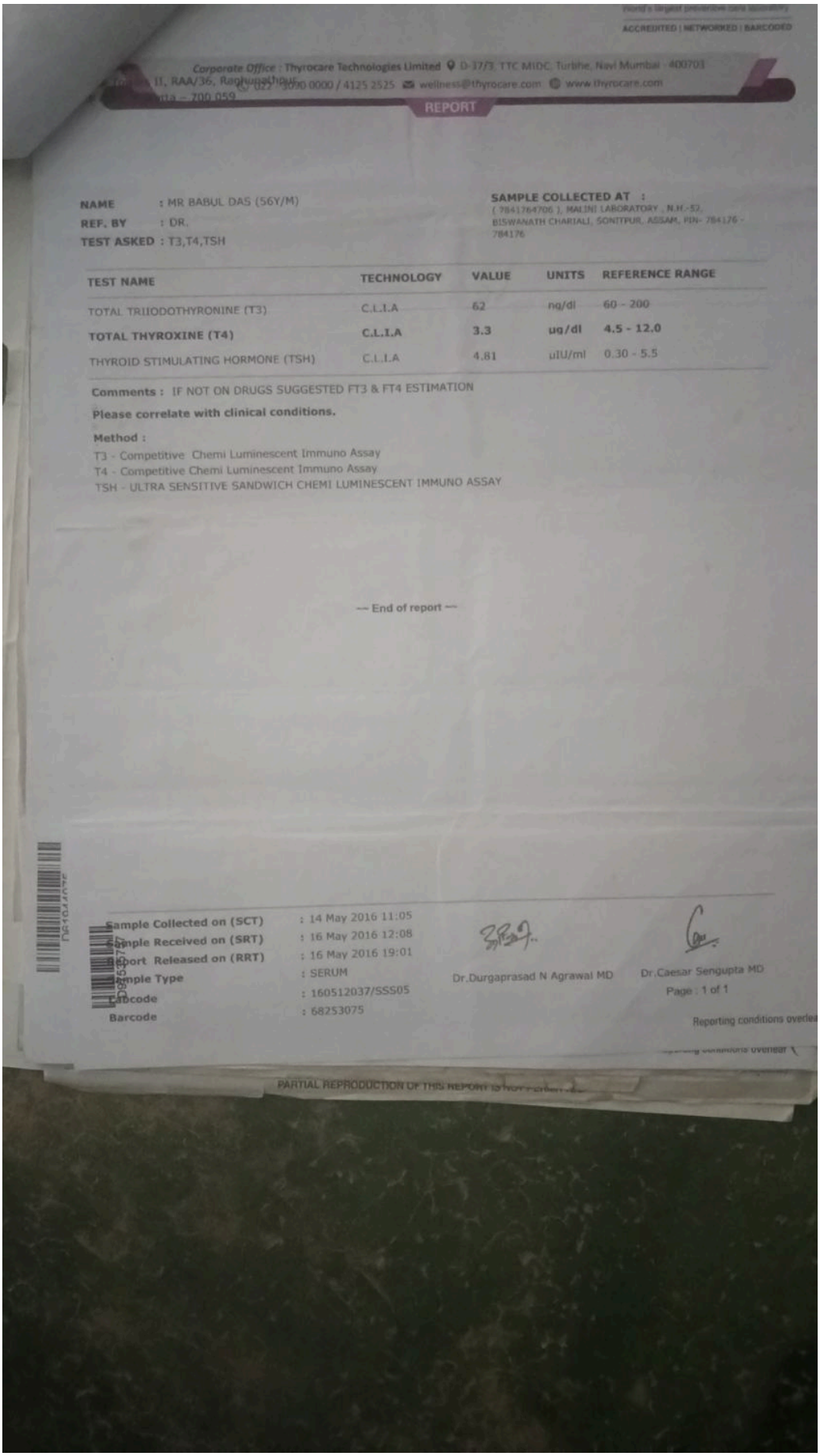
Thyrocare Technologies Ltd. (sample collected at Malini Laboratory, Biswanath Chariali)

Name: Mr Babul Das (56Y/M) · **Ref. by:** Dr. · **Tests:** T3, T4, TSH
Collected (SCT): 14 May 2016 11:05 · **Received:** 16 May 2016 12:08 · **Released:** 16 May 2016 19:01 · Serum · Labcode 160512037/SSS05 · Barcode 68253075

Test	Method	Value	Units	Reference range
Total Triiodothyronine (T3)	C.L.I.A	62	ng/dl	60–200
Total Thyroxine (T4)	C.L.I.A	3.3	µg/dl	4.5–12.0
Thyroid Stimulating Hormone (TSH)	C.L.I.A	4.81	µIU/ml	0.30–5.5

Comments: If not on drugs suggested FT3 & FT4 estimation. Please correlate with clinical conditions.
Signed: Dr. Durgaprasad N Agrawal MD; Dr. Caesar Sengupta MD.

10. 2016-05-14 — Thyroid profile (T3, T4, TSH) · file: 2016-05-14_Thyroid-Profile_Thyrocare.jpg



ACCREDITED | NETWORKED | BARCODED

Corporate Office : Thyrocare Technologies Limited D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400703
022 - 3090 0000 / 4125 2525 wellness@thyrocare.com www.thyrocare.com

REPORT

NAME : MR BABUL DAS (56Y/M)
REF. BY : DR.
TEST ASKED : T3,T4,TSH

SAMPLE COLLECTED AT :
(7841764706), MALINI LABORATORY , N.H.-52, BISWANATH CHARIALI, SONITPUR, ASSAM, PIN- 784176 - 784176

TEST NAME	TECHNOLOGY	VALUE	UNITS	REFERENCE RANGE
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	62	ng/dl	60 - 200
TOTAL THYROXINE (T4)	**C.L.I.A**	**3.3**	**ug/dl**	**4.5 - 12.0**
THYROID STIMULATING HORMONE (TSH)	C.L.I.A	4.81	uIU/ml	0.30 - 5.5

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

Please correlate with clinical conditions.

Method :
T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - ULTRA SENSITIVE SANDWICH CHEMI LUMINESCENT IMMUNO ASSAY

— End of report —

Sample Collected on (SCT) : 14 May 2016 11:05
Sample Received on (SRT) : 16 May 2016 12:08
Report Released on (RRT) : 16 May 2016 19:01
Sample Type : SERUM
Barcode : 160512037/SSS05
Barcode : 68253075

Dr.Durgaprasad N Agrawal MD

Dr.Caesar Sengupta MD

Page : 1 of 1

Reporting conditions overlea

PARTIAL REPRODUCTION OF THIS REPORT

11. 20/08/2019 — Thyroid profile (T3, T4, TSH)

Thyrocare (processed at Kolkata; sample collected at Malini Laboratory, Biswanath Chariali)

Name: Bubul Das (59Y/M) · **Ref. by:** Self · **Tests:** T3-T4-TSH
Collected (SCT): 20 Aug 2019 08:55 · **Received:** 21 Aug 2019 13:04 · **Released:** 21 Aug 2019 15:23 · Serum · Labcode 2108020481/SSS05 · Barcode N3283941

Test	Method	Value	Units	Reference range
Total Triiodothyronine (T3)	C.L.I.A	77	ng/dl	60–200
Total Thyroxine (T4)	C.L.I.A	3.1	µg/dl	4.5–12
Thyroid Stimulating Hormone (TSH)	C.L.I.A	14.02	µIU/ml	0.3–5.5

Comments: If not on drugs suggested FT3 & FT4 estimation. Please correlate with clinical conditions.
Signed: Dr S. Bera MD (Path); Dr. Caesar Sengupta MD (Micro).

11. 2019-08-20 — Thyroid profile (T3, T4, TSH) · file: 2019-08-20_Thyroid-Profile_Thyrocare.jpg



CESSED AT :
Thyrocare
Shree Towers II, RAA/36, Raghunathpur,
VIP Road, Kolkata-700 059

Thyrocare
Think Thyroid. Think Thyrocare.

Corporate Office : Thyrocare Technologies Limited, D-327(), TTC MIDC, Turbhe, Navi Mumbai - 400703
022 - 3090 0000 / 4125 2525 | 8691864086 | wellness@thyrocare.com | www.thyrocare.com

REPORT

NAME : BUBUL DAS (59Y/M)
REF. BY : SELF
TEST ASKED : T3-T4-TSH

SAMPLE COLLECTED AT :
(7841764706),MALINI LABORATORY,N H 52, BISWANATH CHARIALI, SONITPUR, ASSAM, PIN 784176,784176

TEST NAME	TECHNOLOGY	VALUE	UNITS	REFERENCE RANGE
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	77	ng/dl	60-200
TOTAL THYROXINE (T4)	**C.L.I.A**	**3.1**	**µg/dl**	**4.5-12**
THYROID STIMULATING HORMONE (TSH)	**C.L.I.A**	**14.02**	**µIU/ml**	**0.3-5.5**

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

Please correlate with clinical conditions.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - SANDWICH CHEMI LUMINESCENT IMMUNO ASSAY

--- End of report ---

Sample Collected on (SCT) : 20 Aug 2019 08:55
Sample Received on (SRT) : 21 Aug 2019 13:04
Report Released on (RRT) : 21 Aug 2019 15:23
Sample Type : SERUM
Labcode : 2108020481/SSS05
Barcode : N3283941

Dr S.Bera MD(Path)

Dr.Caesar Sengupta MD(Micro)

Page : 1 of 1

PARTIAL REPRODUCTION OF THIS REPORT IS NOT PERMI

12. 18/08/2020 — Pathology report — Random blood glucose

HANDWRITTEN

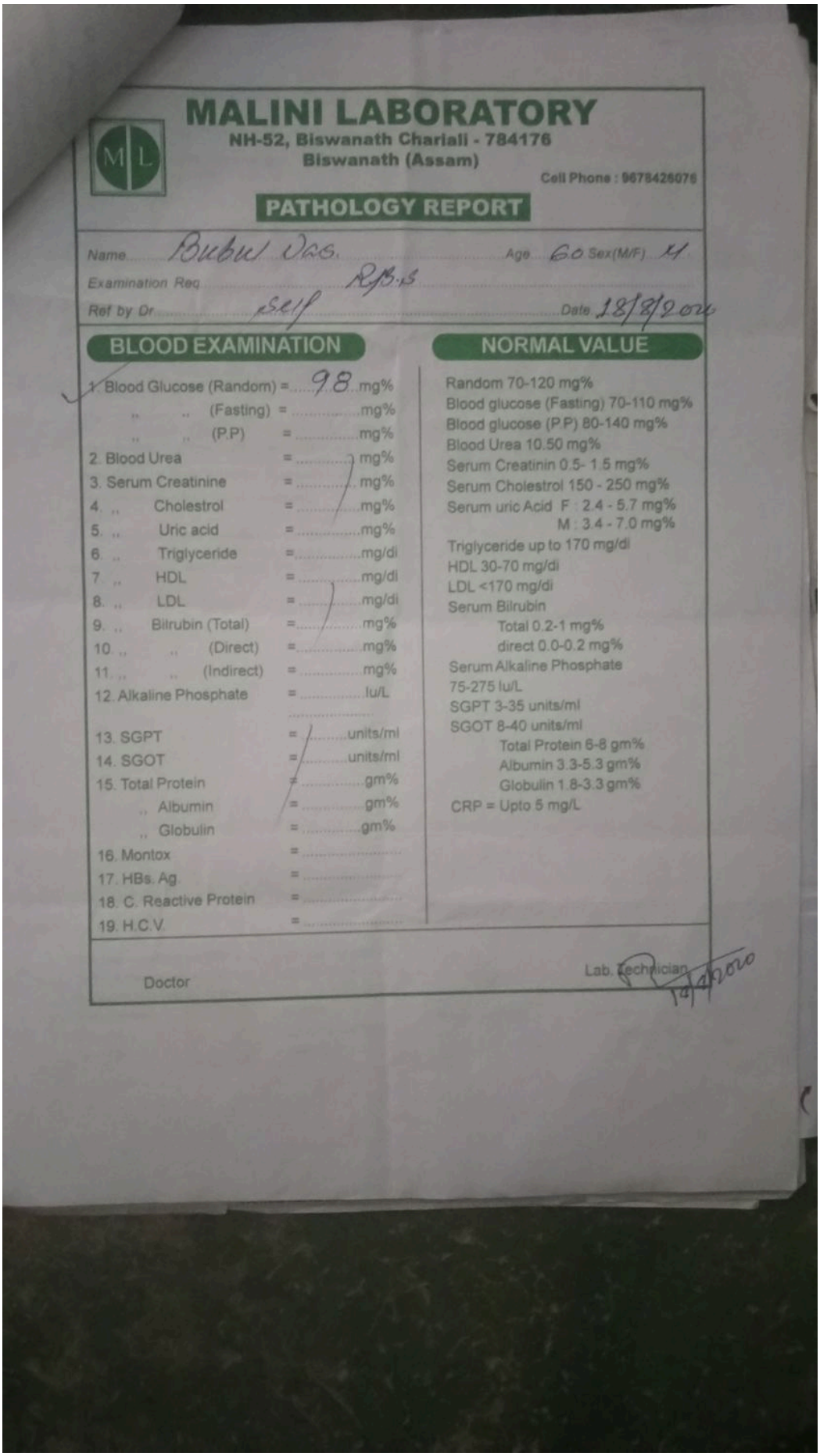
Malini Laboratory, NH-52, Biswanath Chariali — 784176

Name: Bubul Das · **Age:** 60 · **Sex:** M · **Date:** 18/8/2020
Examination required: RBS · **Ref. by Dr.:** Self

Test	Result	Normal value
1. Blood Glucose (Random)	98 mg%	Random 70–120 mg%

All other items on the form (urea, creatinine, cholesterol, uric acid, lipids, bilirubin, SGPT/SGOT, protein, Montox, HBsAg, CRP, HCV) are blank. Lab technician signature dated 18/8/2020.

12. 2020-08-18 — Pathology report — Random blood glucose · file: 2020-08-18_Pathology-Report_Malini-Laboratory.jpg



MALINI LABORATORY

NH-52, Biswanath Chariali - 784176
Biswanath (Assam)

Cell Phone : 9678426076

PATHOLOGY REPORT

Name Bubul Das. Age 60 Sex(M/F) M

Examination Req RBS

Ref by Dr self Date 18/8/2020

BLOOD EXAMINATION

1. Blood Glucose (Random)	= 98	mg%
,, ,, (Fasting)	=	mg%
,, ,, (P.P)	=	mg%
2. Blood Urea	=	mg%
3. Serum Creatinine	=	mg%
4. ,, Cholestrol	=	mg%
5. ,, Uric acid	=	mg%
6. ,, Triglyceride	=	mg/di
7. ,, HDL	=	mg/di
8. ,, LDL	=	mg/di
9. ,, Bilrubin (Total)	=	mg%
10. ,, ,, (Direct)	=	mg%
11. ,, ,, (Indirect)	=	mg%
12. Alkaline Phosphate	=	lu/L
13. SGPT	=	units/ml
14. SGOT	=	units/ml
15. Total Protein	=	gm%
,, Albumin	=	gm%
,, Globulin	=	gm%
16. Montox	=	
17. HBs. Ag	=	
18. C. Reactive Protein	=	
19. H.C.V.	=	

NORMAL VALUE

Random 70-120 mg%
Blood glucose (Fasting) 70-110 mg%
Blood glucose (P.P) 80-140 mg%
Blood Urea 10.50 mg%
Serum Creatinin 0.5- 1.5 mg%
Serum Cholestrol 150 - 250 mg%
Serum uric Acid F : 2.4 - 5.7 mg%
M : 3.4 - 7.0 mg%
Triglyceride up to 170 mg/di
HDL 30-70 mg/di
LDL <170 mg/di
Serum Bilrubin
Total 0.2-1 mg%
direct 0.0-0.2 mg%
Serum Alkaline Phosphate
75-275 lu/L
SGPT 3-35 units/ml
SGOT 8-40 units/ml
Total Protein 6-8 gm%
Albumin 3.3-5.3 gm%
Globulin 1.8-3.3 gm%
CRP = Upto 5 mg/L

Doctor

Lab. Technician 18/8/2020

13. 20/08/2020 — Thyroid profile (T3, T4, TSH) — page 1 of 2

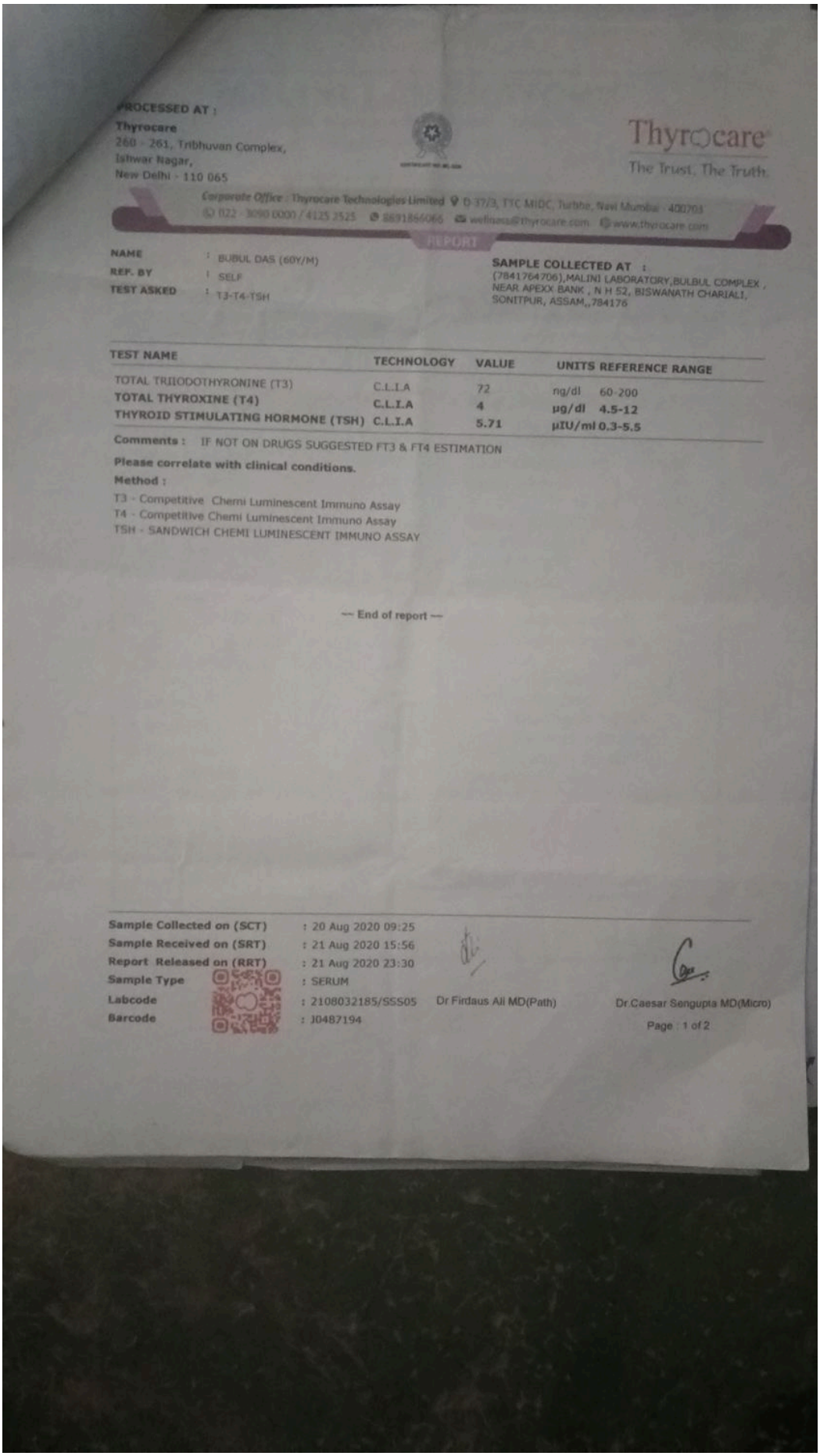
Thyrocare (processed at Delhi; sample collected at Malini Laboratory, Bulbul Complex, Biswanath Chariali)

Name: Bubul Das (60Y/M) · **Ref. by:** Self · **Tests:** T3-T4-TSH
Collected (SCT): 20 Aug 2020 09:25 · **Received:** 21 Aug 2020 15:56 · **Released:** 21 Aug 2020 23:30 · Serum · Labcode 2108032185/SSS05 · Barcode J0487194

Test	Method	Value	Units	Reference range
Total Triiodothyronine (T3)	C.L.I.A	72	ng/dl	60–200
Total Thyroxine (T4)	C.L.I.A	4	µg/dl	4.5–12
Thyroid Stimulating Hormone (TSH)	C.L.I.A	5.71	µIU/ml	0.3–5.5

Comments: If not on drugs suggested FT3 & FT4 estimation. Please correlate with clinical conditions.
Signed: Dr. Firdaus Ali MD (Path); Dr. Caesar Sengupta MD (Micro). Page 1 of 2.

13. 2020-08-20 — Thyroid profile (T3, T4, TSH) — page 1 of 2 · file: 2020-08-20_Thyroid-Profile_Thyrocare_p1of2.jpg



PROCESSED AT :
Thyrocare
260 - 261, Tribhuvan Complex,
Ishwar Nagar,
New Delhi - 110 065

Thyrocare
The Trust. The Truth.

Corporate Office : Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400703
022 - 3090 0000 / 4125 2525 8691866066 wellness@thyrocare.com www.thyrocare.com

REPORT

NAME : BUBUL DAS (60Y/M)
REF. BY : SELF
TEST ASKED : T3-T4-TSH

SAMPLE COLLECTED AT :
(7841764706),MALINI LABORATORY,BULBUL COMPLEX , NEAR APEXX BANK , N H 52, BISWANATH CHARIALI, SONITPUR, ASSAM,,784176

TEST NAME	TECHNOLOGY	VALUE	UNITS	REFERENCE RANGE
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	72	ng/dl	60-200
TOTAL THYROXINE (T4)	**C.L.I.A**	**4**	**µg/dl**	**4.5-12**
THYROID STIMULATING HORMONE (TSH)	**C.L.I.A**	**5.71**	**µIU/ml**	**0.3-5.5**

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

Please correlate with clinical conditions.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - SANDWICH CHEMI LUMINESCENT IMMUNO ASSAY

--- End of report ---

Sample Collected on (SCT) : 20 Aug 2020 09:25
Sample Received on (SRT) : 21 Aug 2020 15:56
Report Released on (RRT) : 21 Aug 2020 23:30
Sample Type : SERUM
Labcode : 2108032185/SSS05
Barcode : J0487194

Dr Firdaus Ali MD(Path)

Dr Caesar Sengupta MD(Micro)

14. 10/04/2023 — TSH, serum (ECLIA) — pages 1 and 2

Dr. Lal PathLabs (processed at Guwahati; collected at M/S Ambika Diagnostic Centre, Tumuki near Tezpur Medical College, Tezpur)

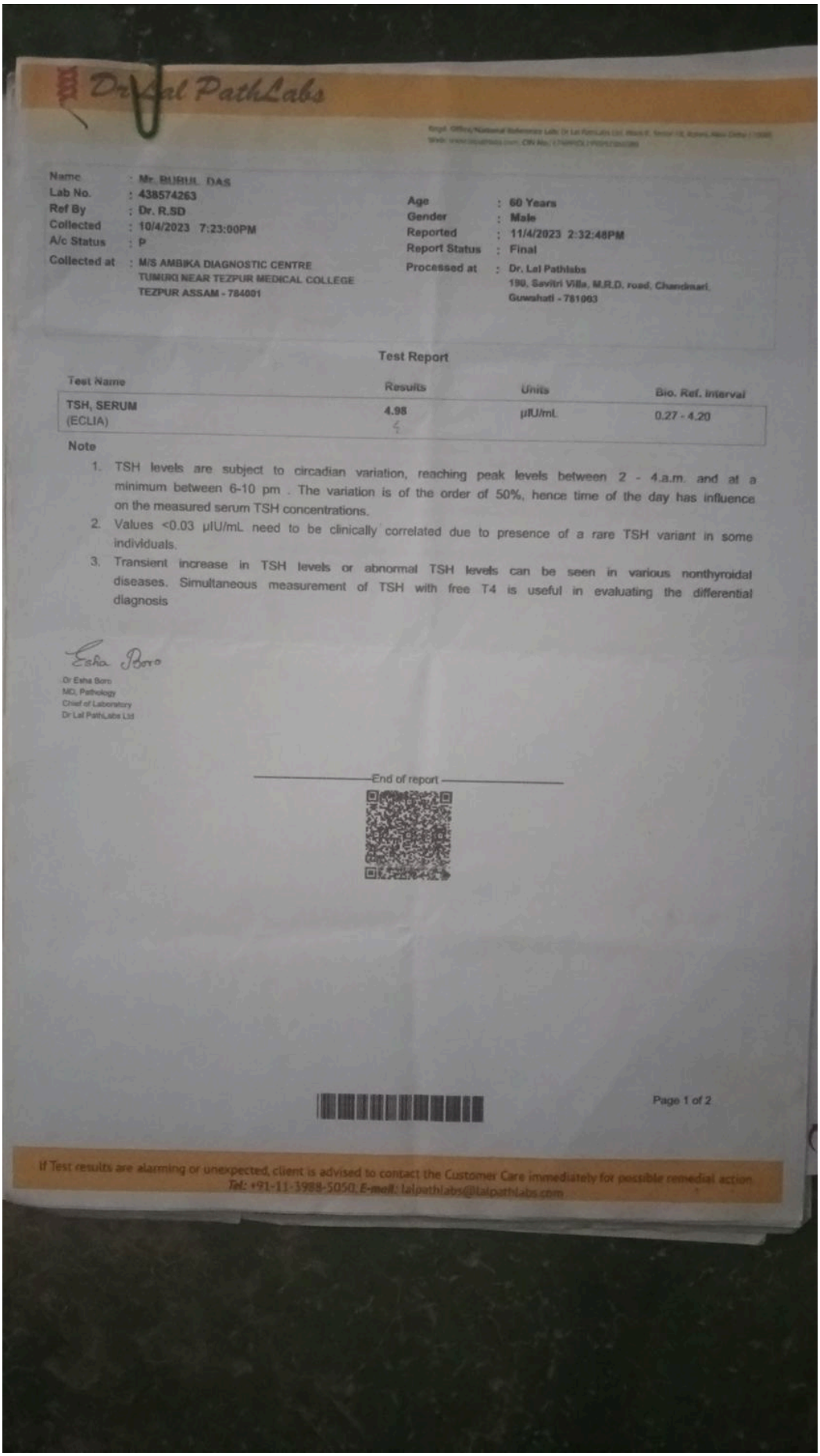
Name: Mr. Bubul Das · **Age:** 60 Years · **Gender:** Male · **Lab No.:** 438574263 · **Ref. by:** Dr. R.SD · A/c Status P
Collected: 10/4/2023 7:23 PM · **Reported:** 11/4/2023 2:32:48 PM · **Status:** Final
Processed at: Dr. Lal PathLabs, 190 Savitri Villa, M.R.D. Road, Chandmari, Guwahati-781003

Test	Result	Units	Bio. Ref. Interval
TSH, SERUM (ECLIA)	4.98	µIU/mL	0.27–4.20

Note: (1) TSH levels are subject to circadian variation, reaching peak levels between 2–4 a.m. and at a minimum between 6–10 pm; the variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations. (2) Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals. (3) Transient increase in TSH levels or abnormal TSH levels can be seen in various nonthyroidal diseases; simultaneous measurement of TSH with free T4 is useful in evaluating the differential diagnosis.

Signed: Dr Esha Boro, MD Pathology, Chief of Laboratory. Page 2 of 2 carries only the printed Important Instructions / disclaimer text.

14. 2023-04-10 — TSH, serum (ECLIA) — pages 1 and 2 (page 1 of 2) · file: 2023-04-10_TSH_Dr-Lal-PathLabs_page1.jpg



Dr Lal PathLabs

Name	: Mr. BUBUL DAS	Age	: 60 Years
Lab No.	: 438574263	Gender	: Male
Ref By	: Dr. R.SD	Reported	: 11/4/2023 2:32:48PM
Collected	: 10/4/2023 7:23:00PM	Report Status	: Final
A/c Status	: P	Processed at	: Dr. Lal Pathlabs 199, Savitri Villa, M.R.D. road, Chandmari, Guwahati - 781003
Collected at	: M/S AMBIKA DIAGNOSTIC CENTRE TUMURI NEAR TEZPUR MEDICAL COLLEGE TEZPUR ASSAM - 784001		

Test Report

Test Name	Results	Units	Bio. Ref. Interval
TSH, SERUM (ECLIA)	4.98	µIU/mL	0.27 - 4.20

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
2. Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.
3. Transient increase in TSH levels or abnormal TSH levels can be seen in various nonthyroidal diseases. Simultaneous measurement of TSH with free T4 is useful in evaluating the differential diagnosis

Esha Boro

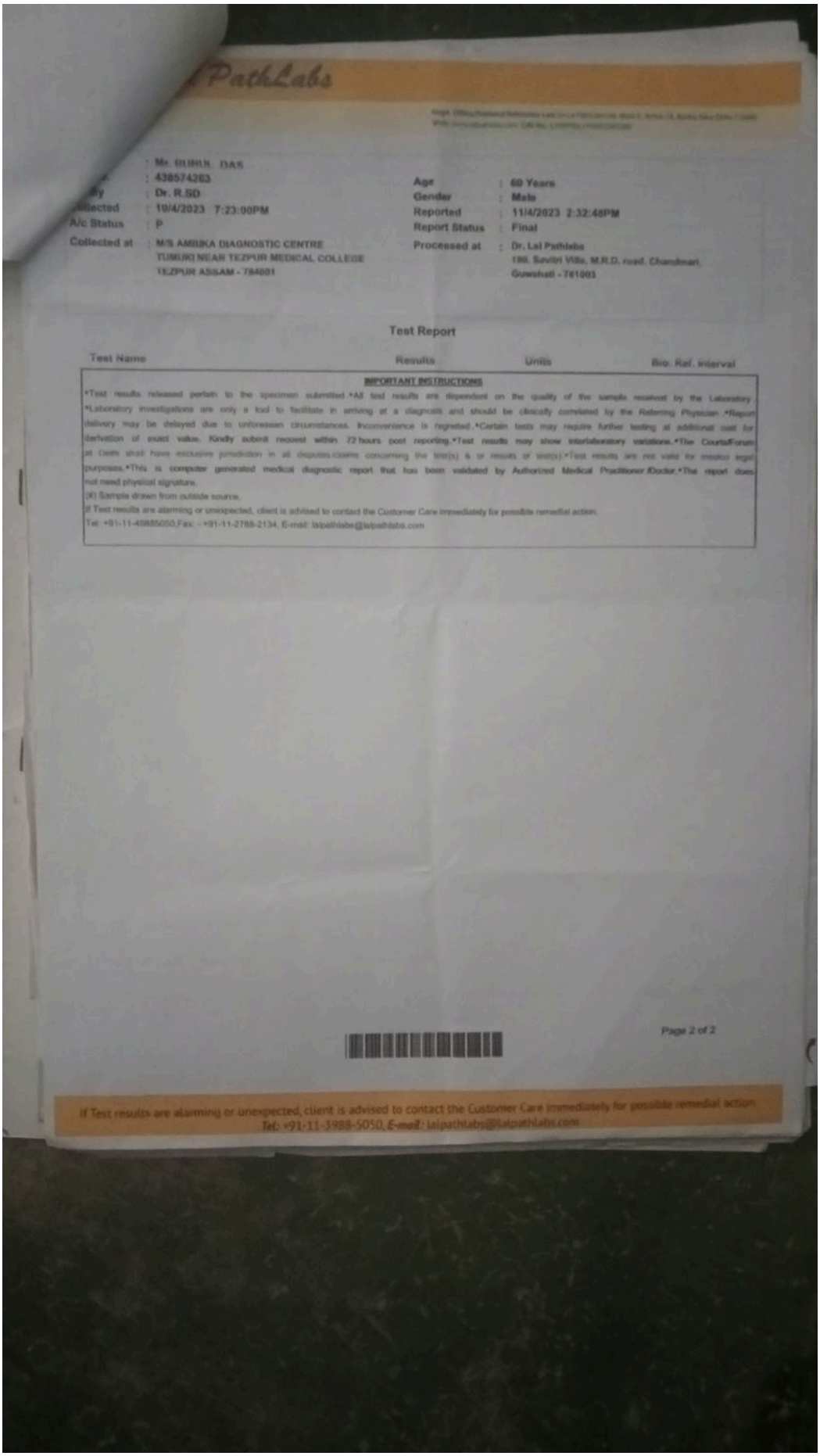
Dr Esha Boro
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

End of report

Page 1 of 2

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
Tel: +91-11-3988-5050, E-mail: lalpathlabs@lalpathlabs.com

14. 2023-04-10 — TSH, serum (ECLIA) — pages 1 and 2 (page 2 of 2) · file: 2023-04-10_TSH_Dr-Lal-PathLabs_page2.jpg



PathLabs

: Mr. BUBUL DAS
: 438574283
: Dr. R.SD
: 10/4/2023 7:23:00PM
A/c Status : P
Collected at : M/S AMBIKA DIAGNOSTIC CENTRE
TUMUKI NEAR TEZPUR MEDICAL COLLEGE
TEZPUR ASSAM - 784001

Age : 60 Years
Gender : Male
Reported : 11/4/2023 2:32:48PM
Report Status : Final
Processed at : Dr. Lal Pathlabs
189, Savitri Villa, M.R.D. road, Chandmari,
Guwahati - 781003

Test Report

Test Name	Results	Units	Bio. Ref. Interval

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted.•All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician.•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.•Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.•Test results may show interlaboratory variations.•The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results or test(s).•Test results are not valid for medico legal purposes.•This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner /Doctor.•The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

Tel: +91-11-49885050,Fax: - +91-11-2788-2134, E-mail: lalpathlabs@lalpathlabs.com

Page 2 of 2

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
Tel: +91-11-3988-5050, E-mail: lalpathlabs@lalpathlabs.com

15. 25/11/2024 — Examination report — Rapid MP test, Typhoid IgG/IgM

Assam Clinical Laboratory, Rangachakua, Sonitpur (Assam) 784182

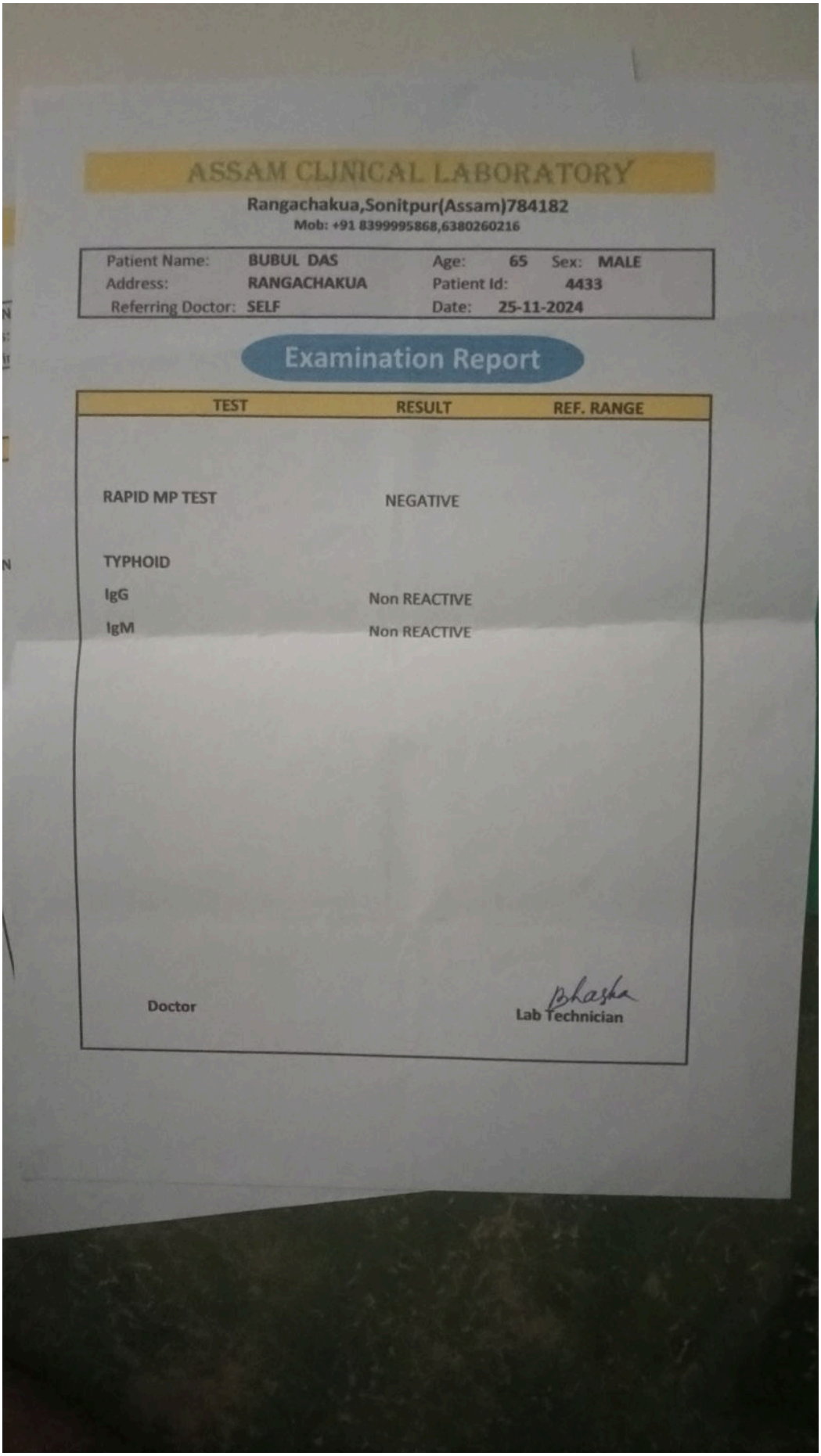
Patient name: BUBUL DAS · **Age:** 65 · **Sex:** MALE · **Patient Id:** 4433 · **Address:** Rangachakua · **Referring doctor:** SELF · **Date:** 25-11-2024

Test	Result
RAPID MP TEST	NEGATIVE
TYPHOID — IgG	Non REACTIVE
TYPHOID — IgM	Non REACTIVE

Signed: Lab Technician (Bhaskar [?]).

> Photograph is upside down in the original; rotated in this PDF.

15. 2024-11-25 — Examination report — Rapid MP test, Typhoid IgG/IgM · file: 2024-11-25_Malaria-Typhoid-Test_Assam-Clinical-Laboratory.jpg



ASSAM CLINICAL LABORATORY

Rangachakua,Sonitpur(Assam)784182

Mob: +91 8399995868,6380260216

Patient Name:	BUBUL DAS	Age: 65	Sex: MALE
Address:	RANGACHAKUA	Patient Id:	4433
Referring Doctor:	SELF	Date:	25-11-2024

Examination Report

TEST	RESULT	REF. RANGE
RAPID MP TEST	NEGATIVE	
TYPHOID		
IgG	Non REACTIVE	
IgM	Non REACTIVE	

Doctor

Bhaska

Lab Technician

16. 25/11/2024 — Examination report — Random blood sugar

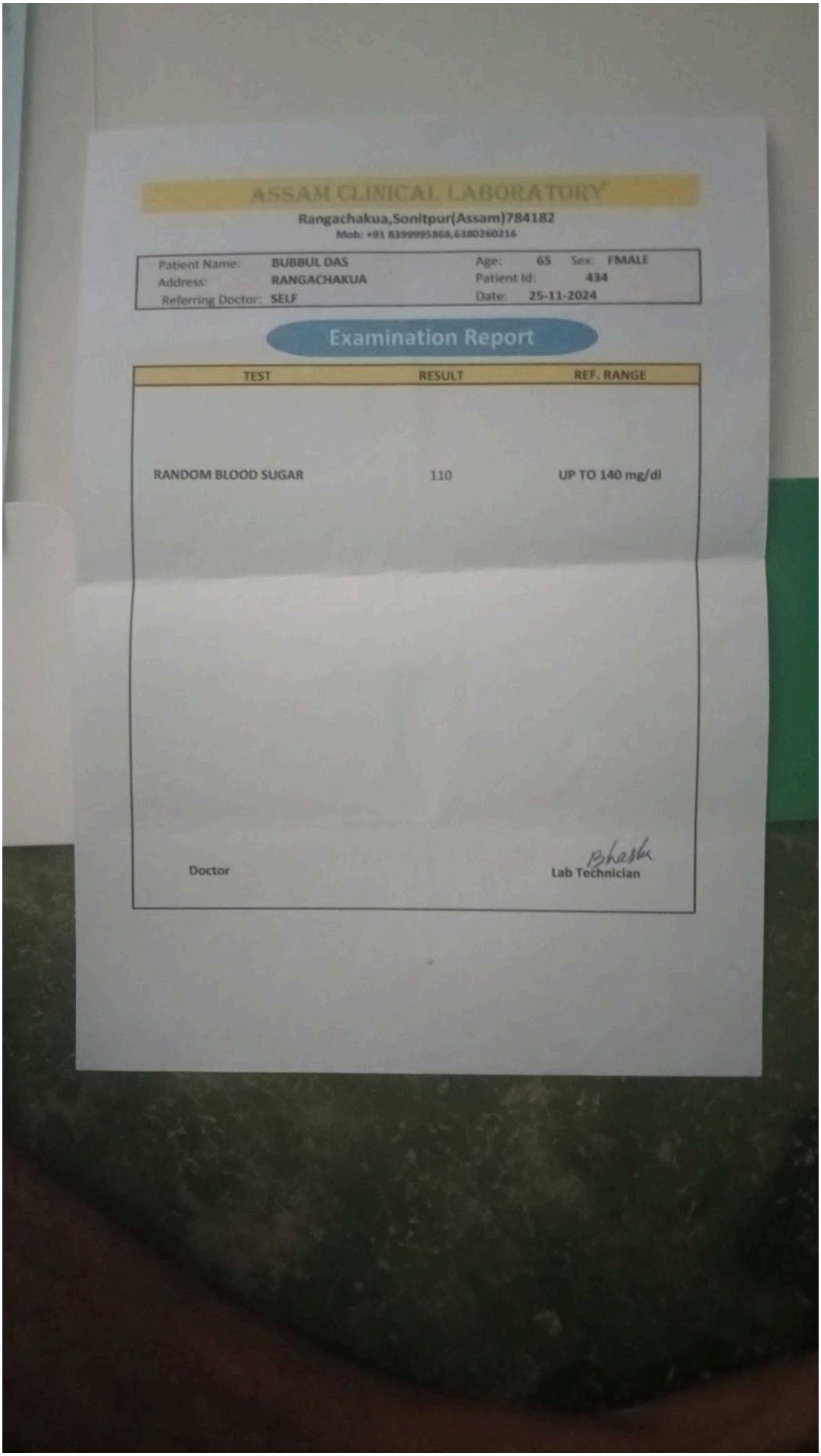
Assam Clinical Laboratory, Rangachakua, Sonitpur (Assam) 784182

Patient name: BUBUL DAS · **Age:** 65 · **Sex:** FMALE [sic, as printed] · **Patient Id:** 434 · **Address:** RANGACHAKUA · **Referring doctor:** SELF · **Date:** 25-11-2024

Test	Result	Ref. range
RANDOM BLOOD SUGAR	110	UP TO 140 mg/dl

Signed: Lab Technician. The report prints sex as "FMALE" and Patient Id 434 (typed lab error; patient is male).

16. 2024-11-25 — Examination report — Random blood sugar · file: 2024-11-25_Random-Blood-Sugar_Assam-Clinical-Laboratory.jpg



ASSAM CLINICAL LABORATORY

Rangachakua,Sonitpur(Assam)784182

Mob: +91 8399995868,6380260216

Patient Name:	BUBBUL DAS	Age: 65 Sex:	FMALE
Address:	RANGACHAKUA	Patient Id:	434
Referring Doctor:	SELF	Date:	25-11-2024

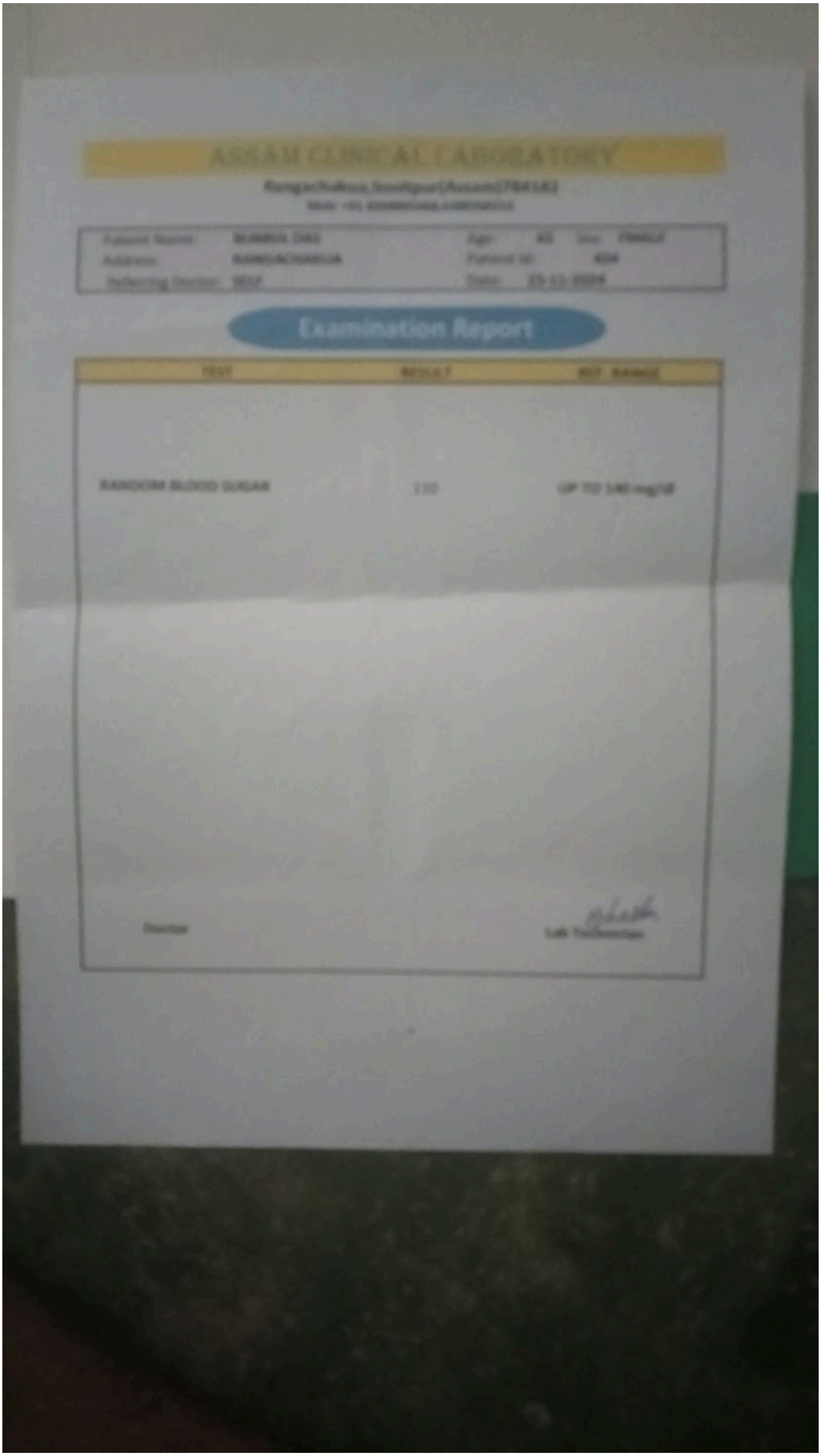
Examination Report

TEST	RESULT	REF. RANGE
RANDOM BLOOD SUGAR	110	UP TO 140 mg/dl

Doctor

Lab Technician

16. 2024-11-25 — Examination report — Random blood sugar (duplicate, blurrier photo of the same report) · file: 2024-11-25_Random-Blood-Sugar_Assam-Clinical-Laboratory_duplicate-photo.jpg



17. 25/11/2024 — Prescription (handwritten) HANDWRITTEN

Dr. Mridurupam Gogoi (M.D.), Medicine Specialist (certified in Diabetology), Senior Medical Officer-1, Kanaklata Civil Hospital, Tezpur — Chamber: Lahkar Drugs House, Civil Hospital Road

Name: Mr. Bubul Das · **Age:** 65 · **Sex:** M · **Date:** 25/11/24 [read; handwriting]
Reg. No.: 17601 · Contact 99542-72854, 93655-42853

Investigation: "UMI" [?] — a note at top right: "Dr. 13.01.24" [?]

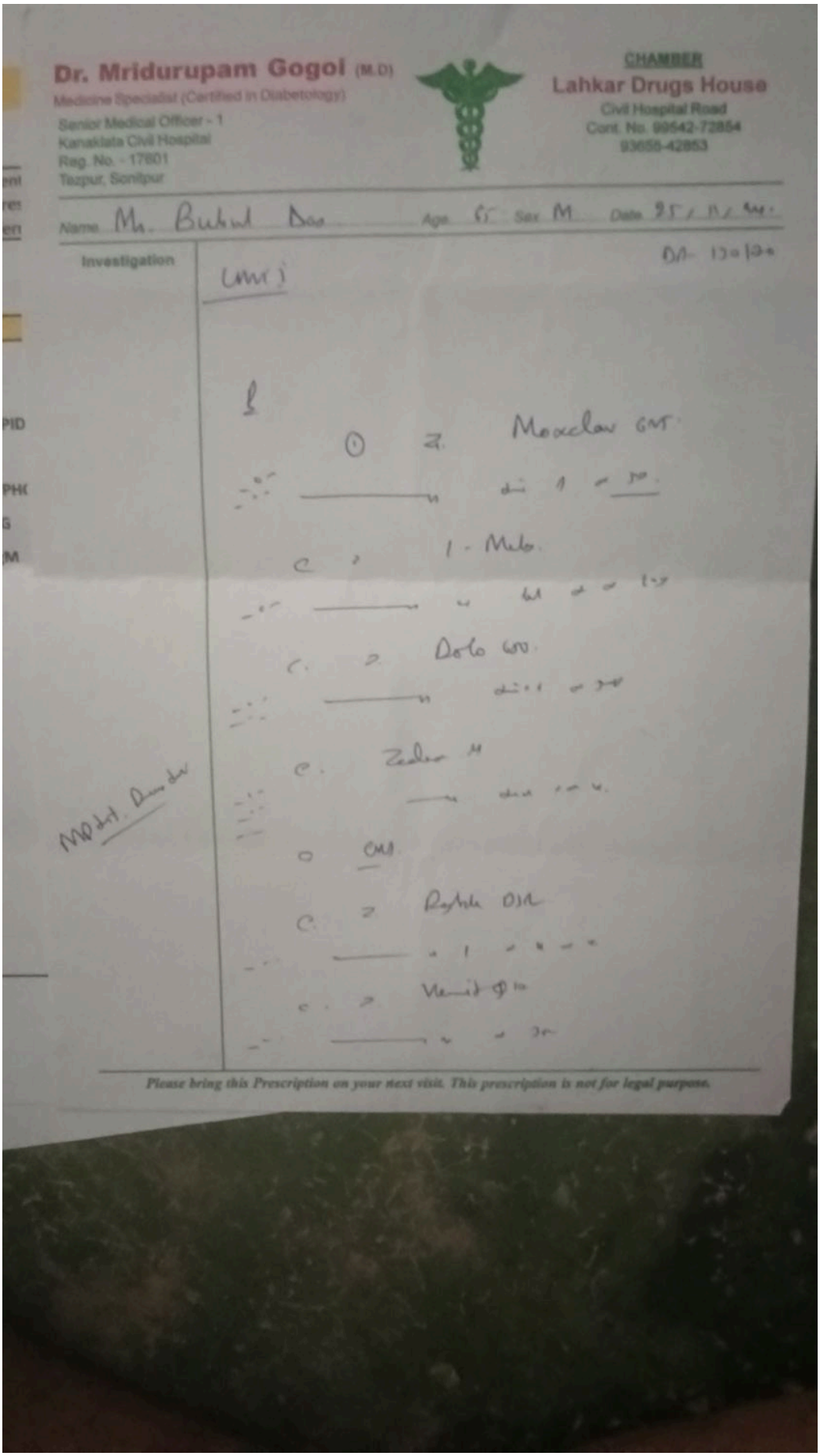
Rx (best effort):

1. Tab Moxclav 625 [?] — … (dose lines illegible)
2. Tab Mala / Melo [?] — 1 …
3. Tab Dolo 650 — …
4. Zeder M / Zedex M [?] …
5. Cm [?]
6. Rejuvil DSR [?] …
7. Vomikind / Vit D3 [?] …

Margin note: "Mohit Dey/Dutta" [?] (handwritten, left margin).

> Handwriting is largely illegible in the photo — treat drug names above as uncertain and verify with the original. Printed footer: "Please bring this prescription on your next visit. This prescription is not for legal purpose."

17. 2024-11-25 — Prescription (handwritten) · file: 2024-11-25_Prescription_Dr-Mridurupam-Gogoi.jpg



Dr. Mridurupam Gogoi (M.D)
Medicine Specialist (Certified in Diabetology)
Senior Medical Officer - 1
Kanaklata Civil Hospital
Reg. No. - 17601
Tezpur, Sonitpur

CHAMBER
Lahkar Drugs House
Civil Hospital Road
Cont. No. 99542-72854
93655-42853

Name Ms. Bulbul Das Age 55 Sex M Date 25/11/24.

Investigation

DA- 12+12+

Rx

1. Moxclav 625 ... 1-0-1 ... 5D.
2. Meb. ... 1-0-1 ... 5D.
3. Dolo 650 ... 1-0-1 ... 5D.
4. Zedex M ... 1-0-1 ... 5D.
5. CM1 ... 0-0-1 ... 5D.
6. Rabifix DSR ... 1-0-0 ... 5D.
7. Vomistop ... 1-0-0 ... 5D.

MD Int. Dm Dr

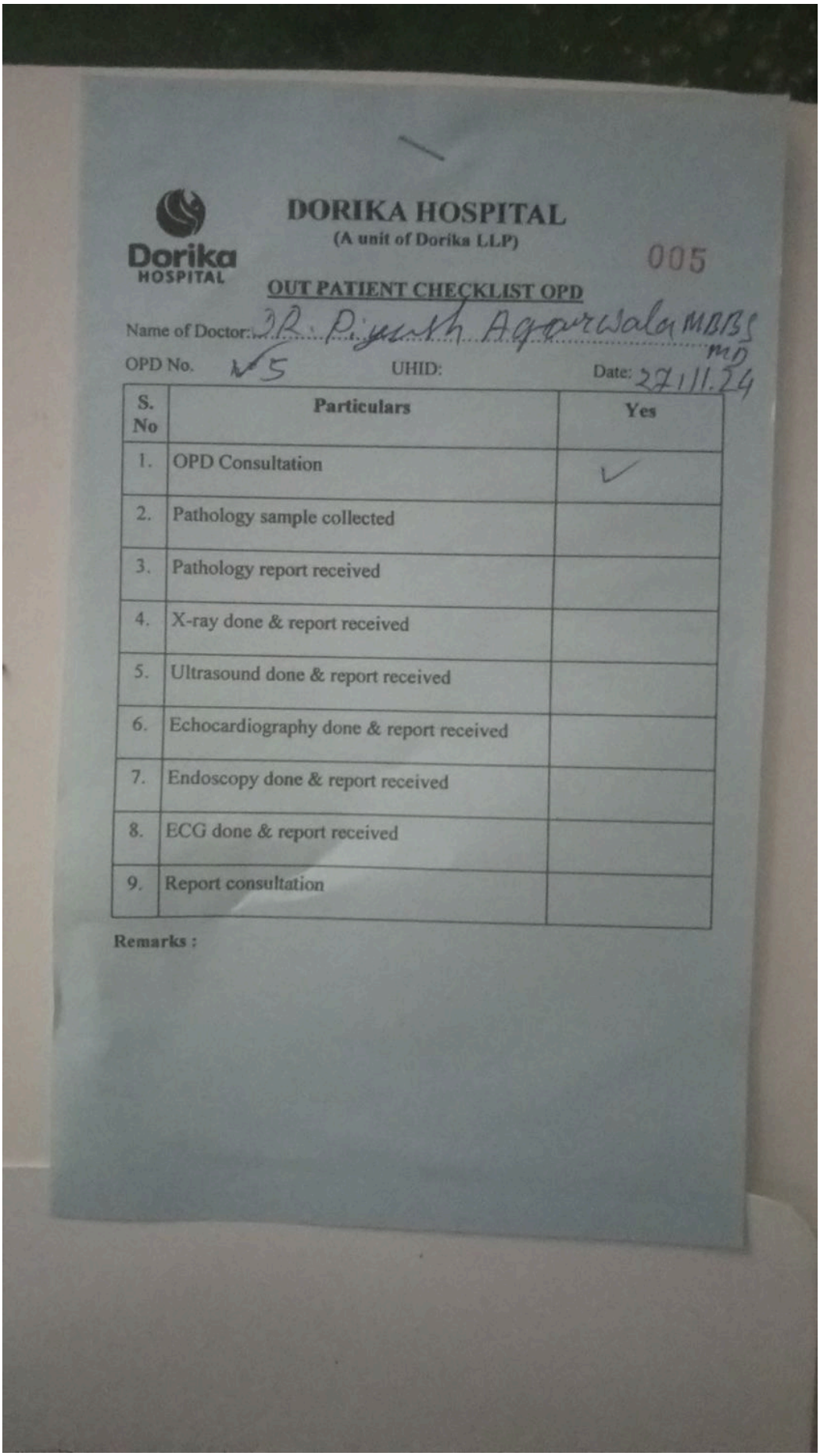
Please bring this Prescription on your next visit. This prescription is not for legal purpose.

18. 27/11/2024 — Out-patient checklist (OPD) HANDWRITTEN

Dorika Hospital (a unit of Dorika LLP), Mazgaon, Tezpur

Name of doctor: Dr. Piyush Agarwala MBBS MD · **OPD No.:** 5 · **Date:** 27.11.24 · Serial stamp 005 · UHID: (blank)

Checklist items: 1 OPD Consultation ✓ (ticked); 2 Pathology sample collected; 3 Pathology report received; 4 X-ray done & report received; 5 Ultrasound; 6 Echocardiography; 7 Endoscopy; 8 ECG; 9 Report consultation. Only item 1 is ticked. Remarks: (blank).



Dorika HOSPITAL

DORIKA HOSPITAL

(A unit of Dorika LLP)

005

OUT PATIENT CHECKLIST OPD

Name of Doctor: DR. Piyush Agarwala MBBS MD

OPD No. 5 UHID: Date: 27/11/24

S. No	Particulars	Yes
1.	OPD Consultation	✓
2.	Pathology sample collected	
3.	Pathology report received	
4.	X-ray done & report received	
5.	Ultrasound done & report received	
6.	Echocardiography done & report received	
7.	Endoscopy done & report received	
8.	ECG done & report received	
9.	Report consultation	

Remarks :

19. 27/11/2024 — OPD prescription / consultation note (handwritten) HANDWRITTEN

Dorika Hospital, NH 715, Mazgaon, Tezpur-784154 — Dr. Piyush Agarwala MBBS MD, Dept. of General Medicine (Regn. No. 19249, AMC)

UHID: DH/9919 · **OP No.:** O-2425/28296 · **Patient:** Mr. BUBUL DAS · **Age:** 64 Y 0 M 0 D · **Gender:** Male · **Date of visit:** 27-NOV-24
Vitals: Weight 57 kg [?] · BP 110/70 · Pulse 80 · SpO2 92 (Temp not filled)
Printed: JD, 27/11/2024 11:07

Handwritten notes (best effort, [?] = uncertain):

- Ref. Dr. Bijoyani Deka [?]
- A/c febrile illness × 3–4 days; ? viral
- H/o irrelevant talking in the morning; having difficulty differentiating dreams from reality; memory (−); no convulsions; chest …; hypertension [?] 50 M [?]
- Investigations noted: CBC; Na/K — Na 128 / K 4.57; TSH — 7.86; CRP — 37.9; urine R/E (✓); X-ray chest PA (printed results from Dorika lab/radiology of 27/11/2024)

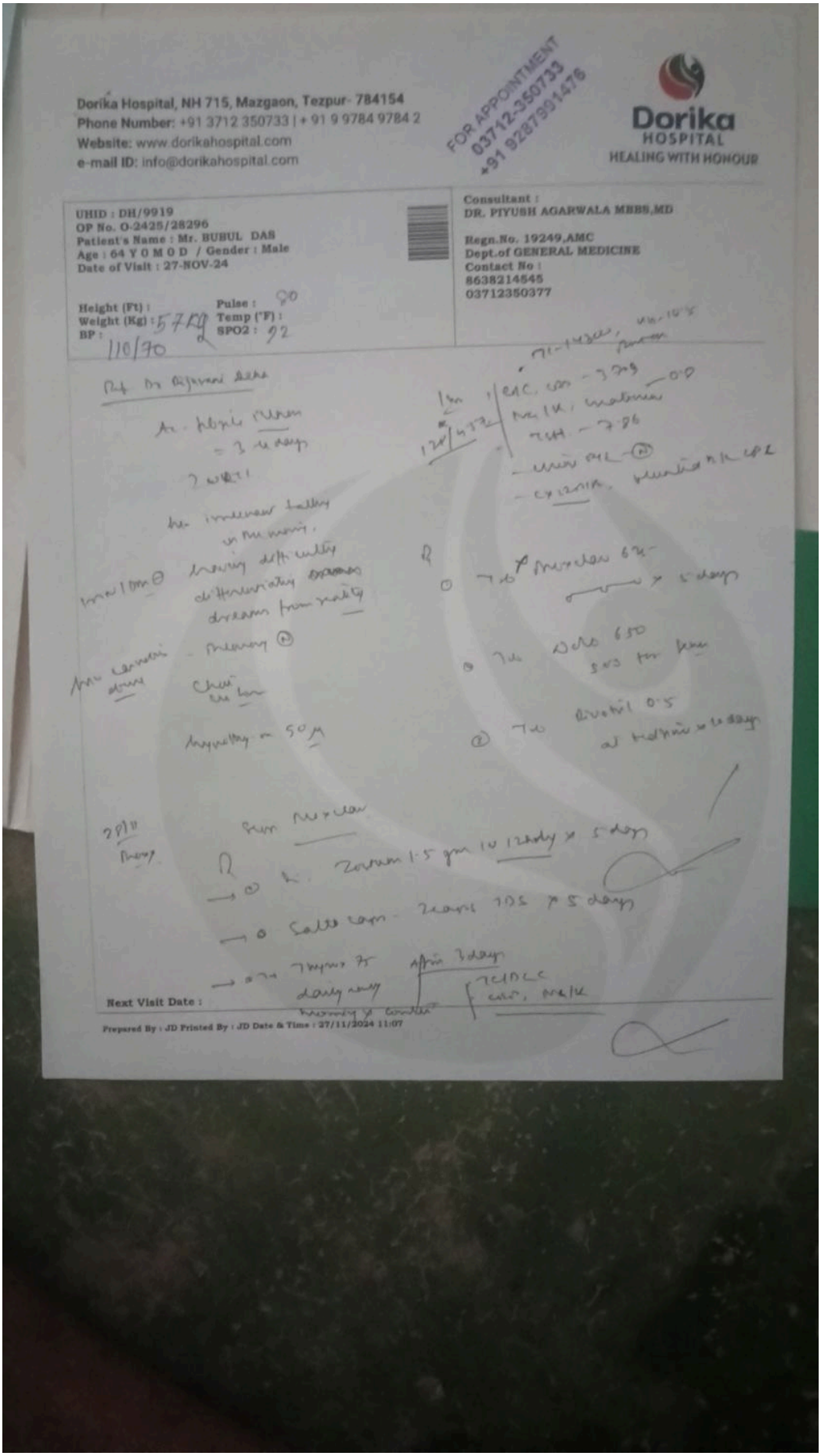
Rx (best effort):

1. Tab Moxclav 625 — × 5 days
2. Tab Dolo 650 — SOS for fever
3. Tab Rivotril 0.5 — at bedtime × 10 days [?]
4. Inj Zoram [?] 1.5 gm IV 12-hrly × 5 days
5. Salt gargle — 2 times / TDS × 5 days
6. Tab Thyronorm 75 — daily morning (empty stomach)

"Advice 3 days" / "review with reports" [?] · "2 Pl'" and margin note "Pt / Mixlan" [?] · Next visit date: (blank).

> Handwriting — best-effort and partly uncertain; verify against the photo on the next page. The Dorika lab values referred to above are transcribed exactly from their own reports in the following pages.

19. 2024-11-27 — OPD prescription / consultation note (handwritten) · file: 2024-11-27_OPD-Prescription_Dorika-Hospital.jpg



Dorika Hospital, NH 715, Mazgaon, Tezpur- 784154
Phone Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

FOR APPOINTMENT
03712-350733
+91 9287991476

Dorika HOSPITAL
HEALING WITH HONOUR

UHID : DH/9919
OP No. O-2425/28296
Patient's Name : Mr. BUBUL DAS
Age : 64 Y 0 M 0 D / Gender : Male
Date of Visit : 27-NOV-24

Height (Ft) :
Weight (Kg) : 57.5
BP : 110/70
Pulse : 80
Temp (°F) :
SPO2 : 92

Consultant :
DR. PIYUSH AGARWALA MBBS,MD

Regn.No. 19249,AMC
Dept.of GENERAL MEDICINE
Contact No :
8638214545
03712350377

Next Visit Date :

Prepared By : JD Printed By : JD Date & Time : 27/11/2024 11:07

20. 27/11/2024 — Complete blood count (CBC)

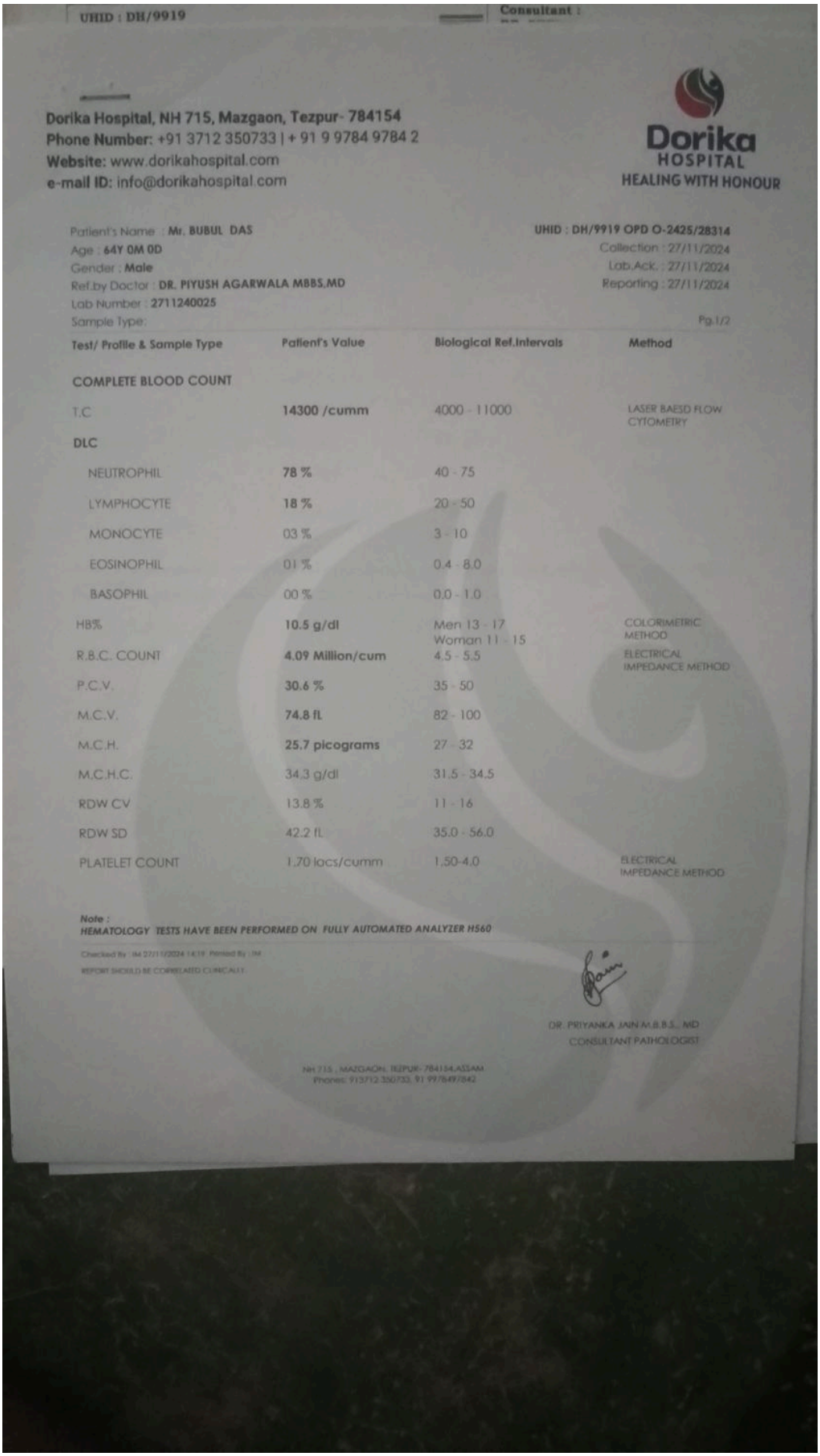
Dorika Hospital laboratory — Dr. Priyanka Jain MBBS MD, Consultant Pathologist

Patient: Mr. BUBUL DAS · **Age:** 64Y 0M 0D · Male · **UHID:** DH/9919 OPD O-2425/28314 · **Lab No.:** 2711240025
Ref. by: Dr. Piyush Agarwala MBBS, MD · **Collection / Lab Ack / Reporting:** 27/11/2024

Test	Patient's value	Biological ref. interval	Method
T.C (Total count)	14300 /cumm	4000–11000	Laser based flow cytometry
Neutrophil	78 %	40–75	
Lymphocyte	18 %	20–50	
Monocyte	03 %	3–10	
Eosinophil	01 %	0.4–8.0	
Basophil	00 %	0.0–1.0	
Hb%	10.5 g/dl	Men 13–17; Women 11–15	Colorimetric
R.B.C. Count	4.09 Million/cum	4.5–5.5	Electrical impedance
P.C.V.	30.6 %	35–50	
M.C.V.	74.8 fl	82–100	
M.C.H.	25.7 picograms	27–32	
M.C.H.C.	34.3 g/dl	31.5–34.5	
RDW CV	13.8 %	11–16	
RDW SD	42.2 fL	35.0–56.0	
Platelet count	1.70 lacs/cumm	1.50–4.0	Electrical impedance

Note: Hematology tests performed on fully automated analyzer H560. Report should be correlated clinically. Page 1/2. Checked by: LM.

20. 2024-11-27 — Complete blood count (CBC) · file: 2024-11-27_CBC_Dorika-Hospital.jpg



UHID : DH/9919

Consultant :

Dorika Hospital, NH 715, Mazgaon, Tezpur- 784154
Phone Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

Dorika HOSPITAL
HEALING WITH HONOUR

Patient's Name : Mr. BUBUL DAS
Age : 64Y 0M 0D
Gender : Male
Ref.by Doctor : DR. PIYUSH AGARWALA MBBS,MD
Lab Number : 2711240025
Sample Type:

UHID : DH/9919 OPD O-2425/28314
Collection : 27/11/2024
Lab.Ack. : 27/11/2024
Reporting : 27/11/2024

Pg.1/2

Test/ Profile & Sample Type	Patient's Value	Biological Ref.Intervals	Method
COMPLETE BLOOD COUNT			
T.C	**14300 /cumm**	4000 - 11000	LASER BAESD FLOW CYTOMETRY
DLC			
NEUTROPHIL	**78 %**	40 - 75	
LYMPHOCYTE	**18 %**	20 - 50	
MONOCYTE	03 %	3 - 10	
EOSINOPHIL	01 %	0.4 - 8.0	
BASOPHIL	00 %	0.0 - 1.0	
HB%	**10.5 g/dl**	Men 13 - 17 Woman 11 - 15	COLORIMETRIC METHOD
R.B.C. COUNT	**4.09 Million/cum**	4.5 - 5.5	ELECTRICAL IMPEDANCE METHOD
P.C.V.	**30.6 %**	35 - 50	
M.C.V.	**74.8 fL**	82 - 100	
M.C.H.	**25.7 picograms**	27 - 32	
M.C.H.C.	34.3 g/dl	31.5 - 34.5	
RDW CV	13.8 %	11 - 16	
RDW SD	42.2 fL	35.0 - 56.0	
PLATELET COUNT	1.70 lacs/cumm	1.50-4.0	ELECTRICAL IMPEDANCE METHOD

Note :
HEMATOLOGY TESTS HAVE BEEN PERFORMED ON FULLY AUTOMATED ANALYZER H560

Checked By : IM 27/11/2024 14:19 Printed By : IM

REPORT SHOULD BE CORRELATED CLINICALLY

DR. PRIYANKA JAIN M.B.B.S., MD
CONSULTANT PATHOLOGIST

NH 715 , MAZGAON, TEZPUR- 784154,ASSAM
Phones: 913712 350733, 91 9978497842

21. 27/11/2024 — Biochemistry — Sodium, Potassium, Creatinine, CRP, TSH

Dorika Hospital laboratory — Dr. Priyanka Jain MBBS MD, Consultant Pathologist

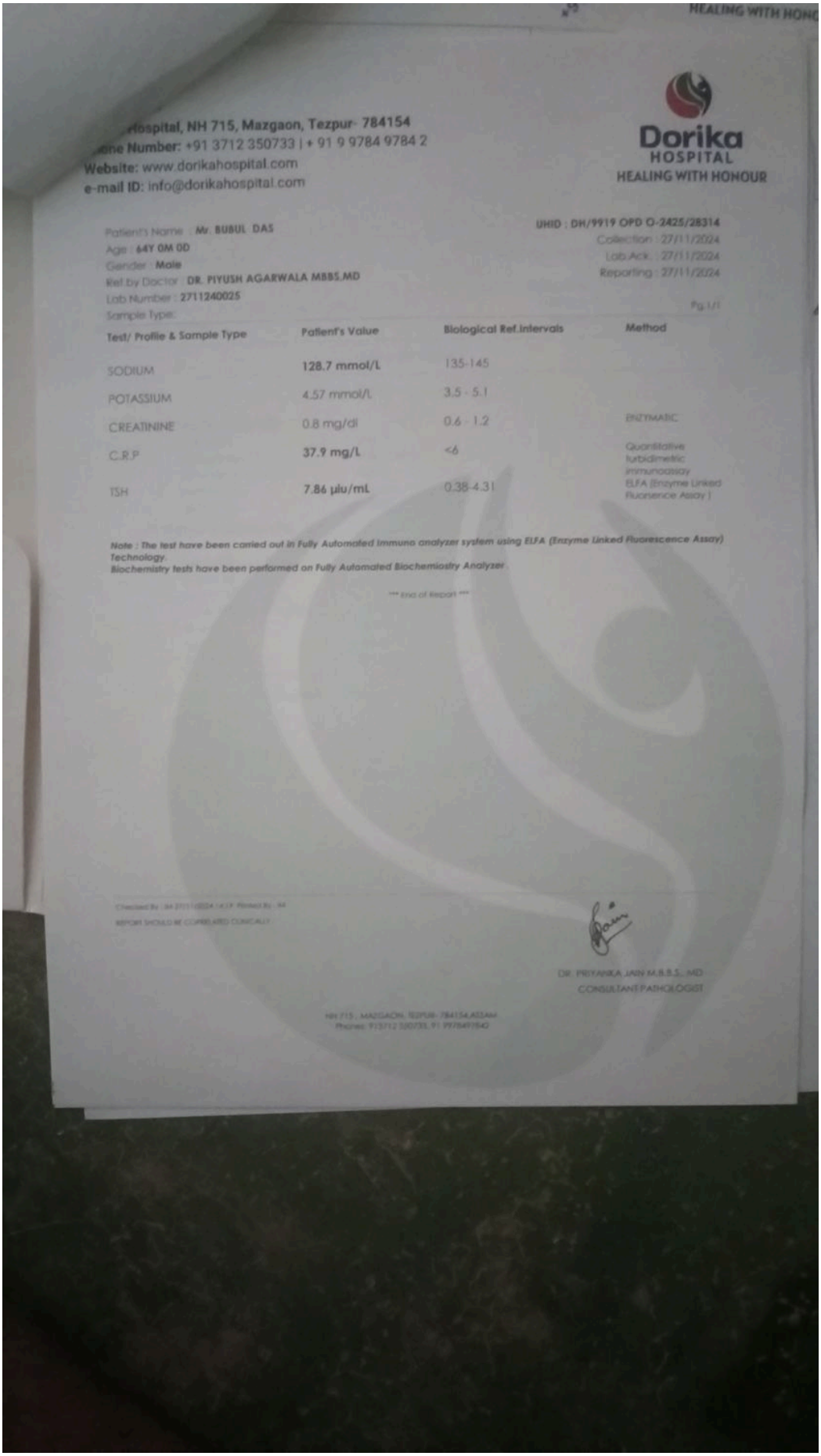
Patient: Mr. BUBUL DAS · **Age:** 64Y 0M 0D · Male · **UHID:** DH/9919 OPD O-2425/28314 · **Lab No.:** 2711240025

Ref. by: Dr. Piyush Agarwala MBBS, MD · Collection / Lab Ack / Reporting: 27/11/2024 · Pg 1/1

Test	Patient's value	Biological ref. interval	Method
SODIUM	128.7 mmol/L	135–145	
POTASSIUM	4.57 mmol/L	3.5–5.1	
CREATININE	0.8 mg/dl	0.6–1.2	Enzymatic
C.R.P	37.9 mg/L	<6	Quantitative turbidimetric immunoassay
TSH	7.86 µIU/mL	0.38–4.31	ELFA (Enzyme Linked Fluorescence Assay)

Note: tests carried out on fully automated immuno-analyzer system using ELFA; biochemistry tests on a fully automated biochemistry analyzer.

21. 2024-11-27 — Biochemistry — Sodium, Potassium, Creatinine, CRP, TSH · file: 2024-11-27_Biochemistry-Sodium-Potassium-Creatinine-CRP-TSH_Dorika-Hospital.jpg



HEALING WITH HONO

Dorika
HOSPITAL
HEALING WITH HONOUR

...rlospital, NH 715, Mazgaon, Tezpur- 784154
...one Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

Patient's Name : Mr. BUBUL DAS
Age : 64Y 0M 0D
Gender : Male
Ref by Doctor : DR. PIYUSH AGARWALA MBBS,MD
Lab Number : 2711240025
Sample Type:

UHID : DH/9919 OPD O-2425/28314
Collection : 27/11/2024
Lab Ack. : 27/11/2024
Reporting : 27/11/2024

Pg.1/1

Test/ Profile & Sample Type	Patient's Value	Biological Ref.Intervals	Method
SODIUM	128.7 mmol/L	135-145	
POTASSIUM	4.57 mmol/L	3.5 - 5.1	
CREATININE	0.8 mg/dl	0.6 - 1.2	ENZYMATIC
C.R.P	37.9 mg/L	<6	Quantitative turbidimetric immunoassay
TSH	7.86 µlu/mL	0.38-4.31	ELFA (Enzyme Linked Fluorence Assay)

Note : The test have been carried out in Fully Automated Immuno analyzer system using ELFA (Enzyme Linked Fluorescence Assay) Technology.
Biochemistry tests have been performed on Fully Automated Biochemiostry Analyzer.

*** End of Report ***

DR. PRIYANKA JAIN M.B.B.S., MD
CONSULTANT PATHOLOGIST

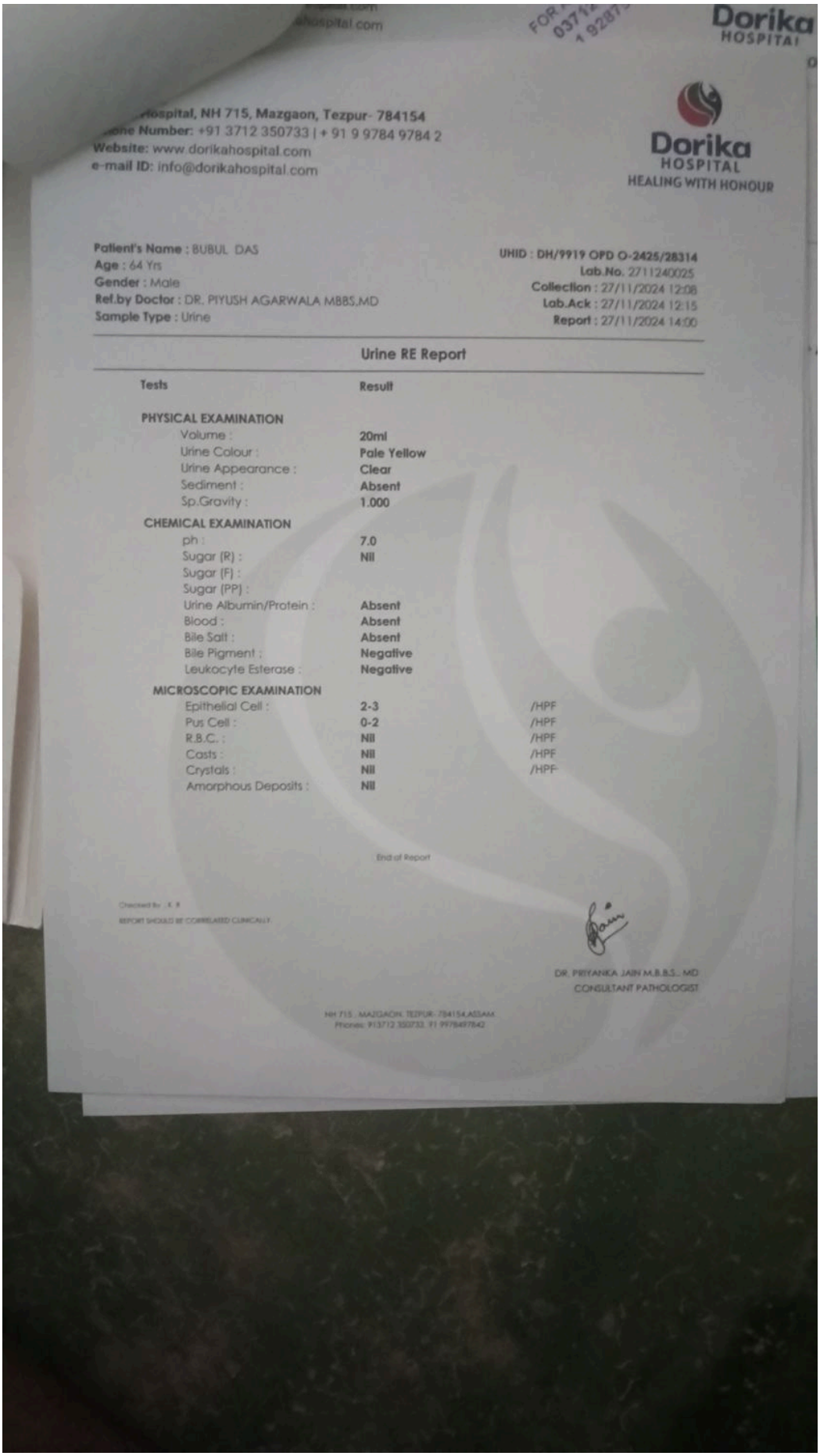
22. 27/11/2024 — Urine R/E report

Dorika Hospital laboratory — Dr. Priyanka Jain MBBS MD, Consultant Pathologist

Patient: BUBUL DAS · **Age:** 64 Yrs · Male · **UHID:** DH/9919 OPD O-2425/28314 · **Lab No.:** 2711240025
Ref. by: Dr. Piyush Agarwala MBBS, MD · Collection 27/11/2024 12:08 · Lab Ack 12:15 · Report 14:00 · Sample: Urine

Test	Result
Physical	
Volume	20 ml
Urine colour	Pale Yellow
Urine appearance	Clear
Sediment	Absent
Sp. gravity	1.000
Chemical	
pH	7.0
Sugar (R)	Nil
Urine albumin/protein	Absent
Blood	Absent
Bile salt	Absent
Bile pigment	Negative
Leukocyte esterase	Negative
Microscopic	
Epithelial cell	2–3 /HPF
Pus cell	0–2 /HPF
R.B.C.	Nil /HPF
Casts	Nil /HPF
Crystals	Nil /HPF
Amorphous deposits	Nil

22. 2024-11-27 — Urine R/E report · file: 2024-11-27_Urine-RE_Dorika-Hospital.jpg



Dorika HOSPITAL

Dorika HOSPITAL
HEALING WITH HONOUR

Hospital, NH 715, Mazgaon, Tezpur- 784154
Phone Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

Patient's Name : BUBUL DAS
Age : 64 Yrs
Gender : Male
Ref.by Doctor : DR. PIYUSH AGARWALA MBBS,MD
Sample Type : Urine

UHID : DH/9919 OPD O-2425/28314
Lab.No. 2711240025
Collection : 27/11/2024 12:08
Lab.Ack : 27/11/2024 12:15
Report : 27/11/2024 14:00

Urine RE Report

Tests	Result	
PHYSICAL EXAMINATION		
Volume :	**20ml**	
Urine Colour :	**Pale Yellow**	
Urine Appearance :	**Clear**	
Sediment :	**Absent**	
Sp.Gravity :	**1.000**	
CHEMICAL EXAMINATION		
ph :	**7.0**	
Sugar (R) :	**Nil**	
Sugar (F) :		
Sugar (PP) :		
Urine Albumin/Protein :	**Absent**	
Blood :	**Absent**	
Bile Salt :	**Absent**	
Bile Pigment :	**Negative**	
Leukocyte Esterase :	**Negative**	
MICROSCOPIC EXAMINATION		
Epithelial Cell :	**2-3**	/HPF
Pus Cell :	**0-2**	/HPF
R.B.C. :	**Nil**	/HPF
Casts :	**Nil**	/HPF
Crystals :	**Nil**	/HPF
Amorphous Deposits :	**Nil**	

End of Report

Checked By : K. B

REPORT SHOULD BE CORRELATED CLINICALLY

DR. PRIYANKA JAIN M.B.B.S., MD
CONSULTANT PATHOLOGIST

NH 715 , MAZGAON, TEZPUR- 784154,ASSAM
Phones: 913712 350733, 91 9978497842

23. 27/11/2024 — X-ray chest (PA view) — report

Dorika Hospital — Dr. A. K. Sarkar, MD, Radiologist

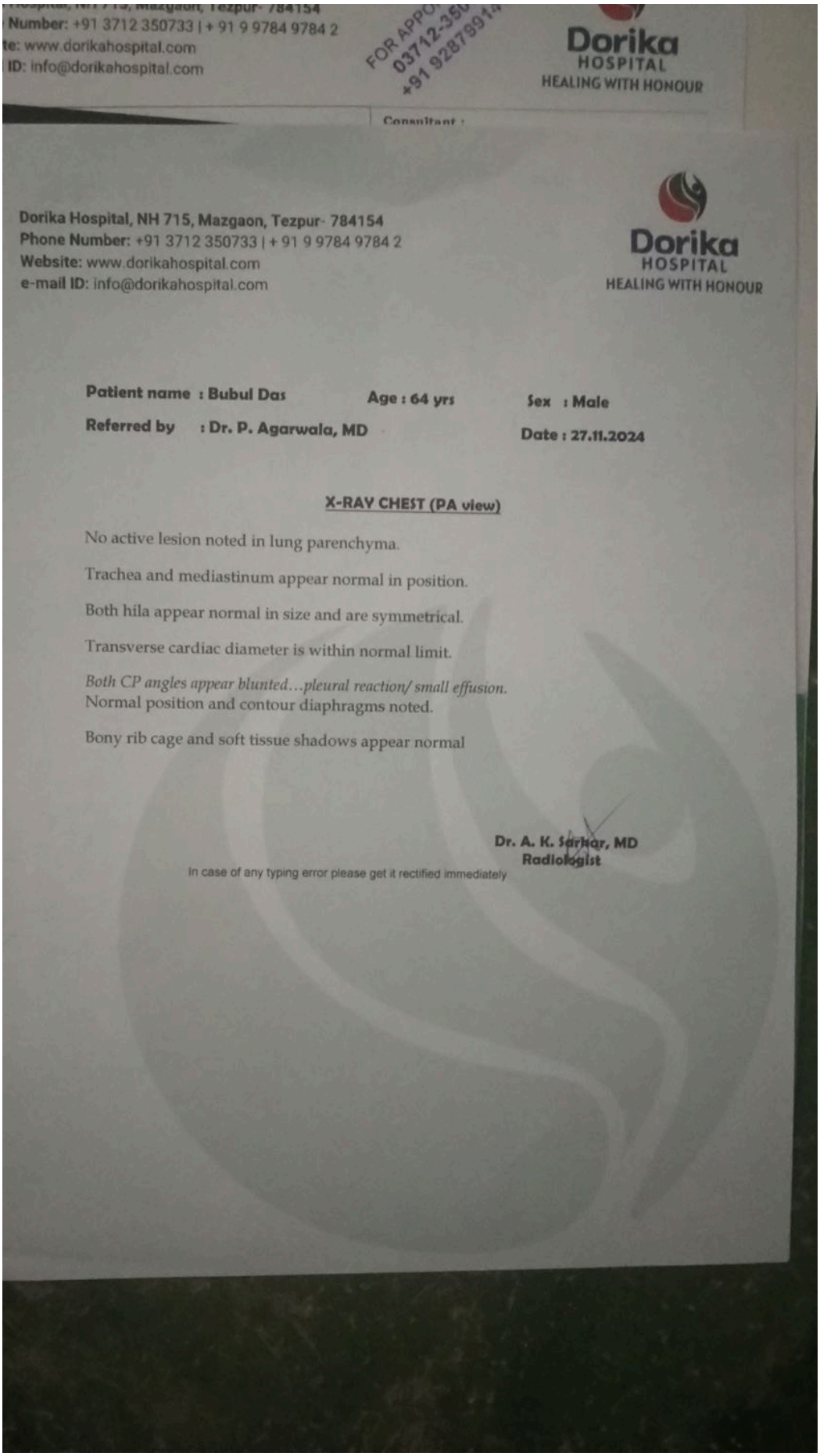
Patient name: Bubul Das · **Age:** 64 yrs · **Sex:** Male · **Referred by:** Dr. P. Agarwala, MD · **Date:** 27.11.2024 · UHID DH/9919

X-RAY CHEST (PA view)

- No active lesion noted in lung parenchyma.
- Trachea and mediastinum appear normal in position.
- Both hila appear normal in size and are symmetrical.
- Transverse cardiac diameter is within normal limit.
- Both CP angles appear blunted… pleural reaction / small effusion.
- Normal position and contour diaphragms noted.
- Bony rib cage and soft tissue shadows appear normal.

Dr. A. K. Sarkar, MD, Radiologist. "In case of any typing error please get it rectified immediately."

23. 2024-11-27 — X-ray chest (PA view) — report · file: 2024-11-27_Chest-Xray-Report_Dorika-Hospital.jpg



Number: +91 3712 350733 | + 91 9 9784 9784 2
te: www.dorikahospital.com
ID: info@dorikahospital.com

FOR APPO
03712-350
+91 92879914

Dorika
HOSPITAL
HEALING WITH HONOUR

Consultant :

Dorika Hospital, NH 715, Mazgaon, Tezpur- 784154
Phone Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

Dorika
HOSPITAL
HEALING WITH HONOUR

Patient name : Bubul Das **Age : 64 yrs** **Sex : Male**

Referred by : Dr. P. Agarwala, MD **Date : 27.11.2024**

X-RAY CHEST (PA view)

No active lesion noted in lung parenchyma.

Trachea and mediastinum appear normal in position.

Both hila appear normal in size and are symmetrical.

Transverse cardiac diameter is within normal limit.

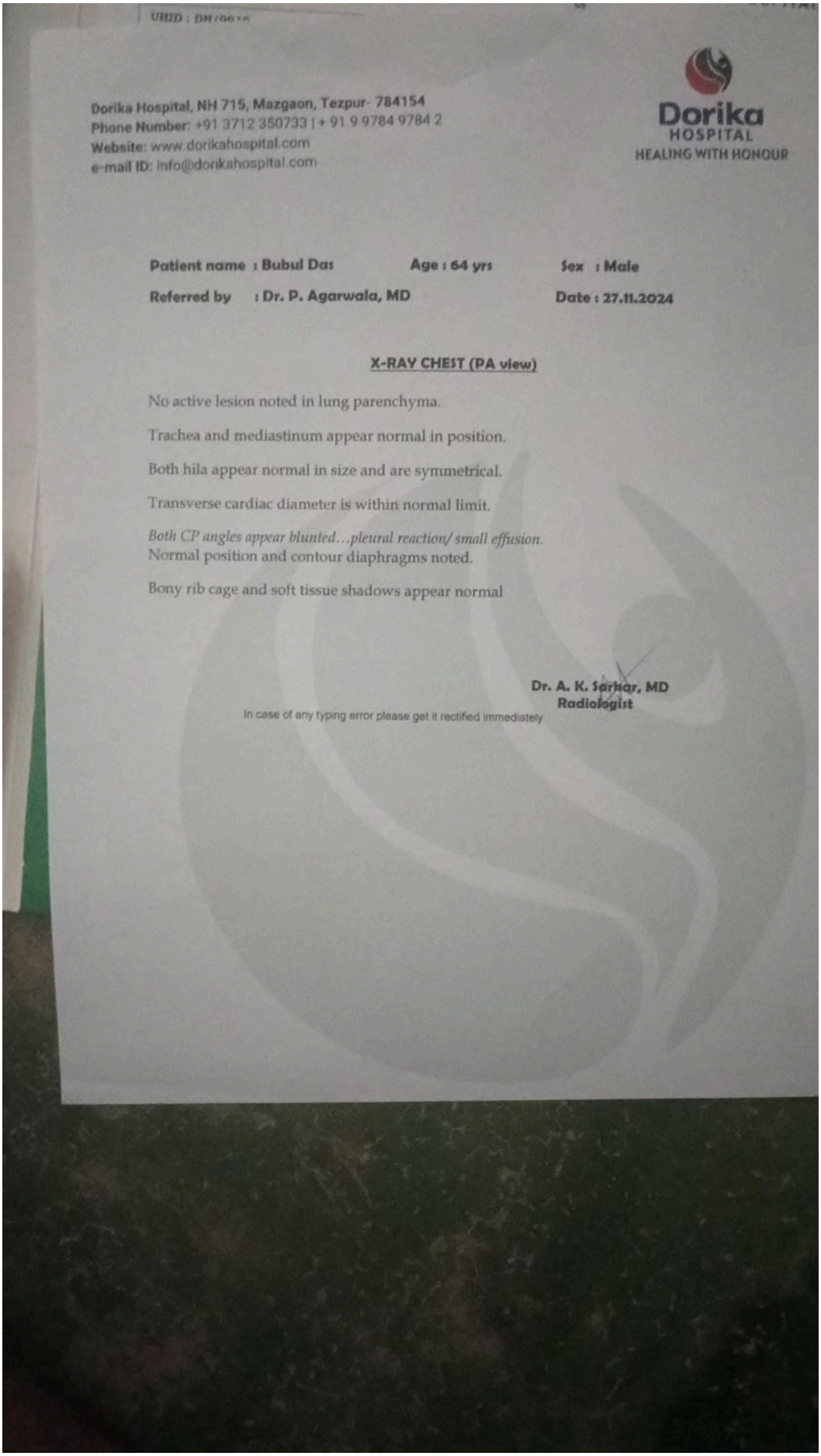
Both CP angles appear blunted…pleural reaction/ small effusion.
Normal position and contour diaphragms noted.

Bony rib cage and soft tissue shadows appear normal

Dr. A. K. Sarkar, MD
Radiologist

In case of any typing error please get it rectified immediately

23. 2024-11-27 — X-ray chest (PA view) — report (second photo of the same report) · file: 2024-11-27_Chest-Xray-Report_Dorika-Hospital_duplicate-photo.jpg



UHID : DH/66**

Dorika Hospital, NH 715, Mazgaon, Tezpur- 784154
Phone Number: +91 3712 350733 | + 91 9 9784 9784 2
Website: www.dorikahospital.com
e-mail ID: info@dorikahospital.com

Dorika
HOSPITAL
HEALING WITH HONOUR

Patient name : Bubul Das **Age : 64 yrs** **Sex : Male**

Referred by : Dr. P. Agarwala, MD **Date : 27.11.2024**

X-RAY CHEST (PA view)

No active lesion noted in lung parenchyma.

Trachea and mediastinum appear normal in position.

Both hila appear normal in size and are symmetrical.

Transverse cardiac diameter is within normal limit.

Both CP angles appear blunted…pleural reaction/ small effusion.
Normal position and contour diaphragms noted.

Bony rib cage and soft tissue shadows appear normal

Dr. A. K. Sarkar, MD
Radiologist

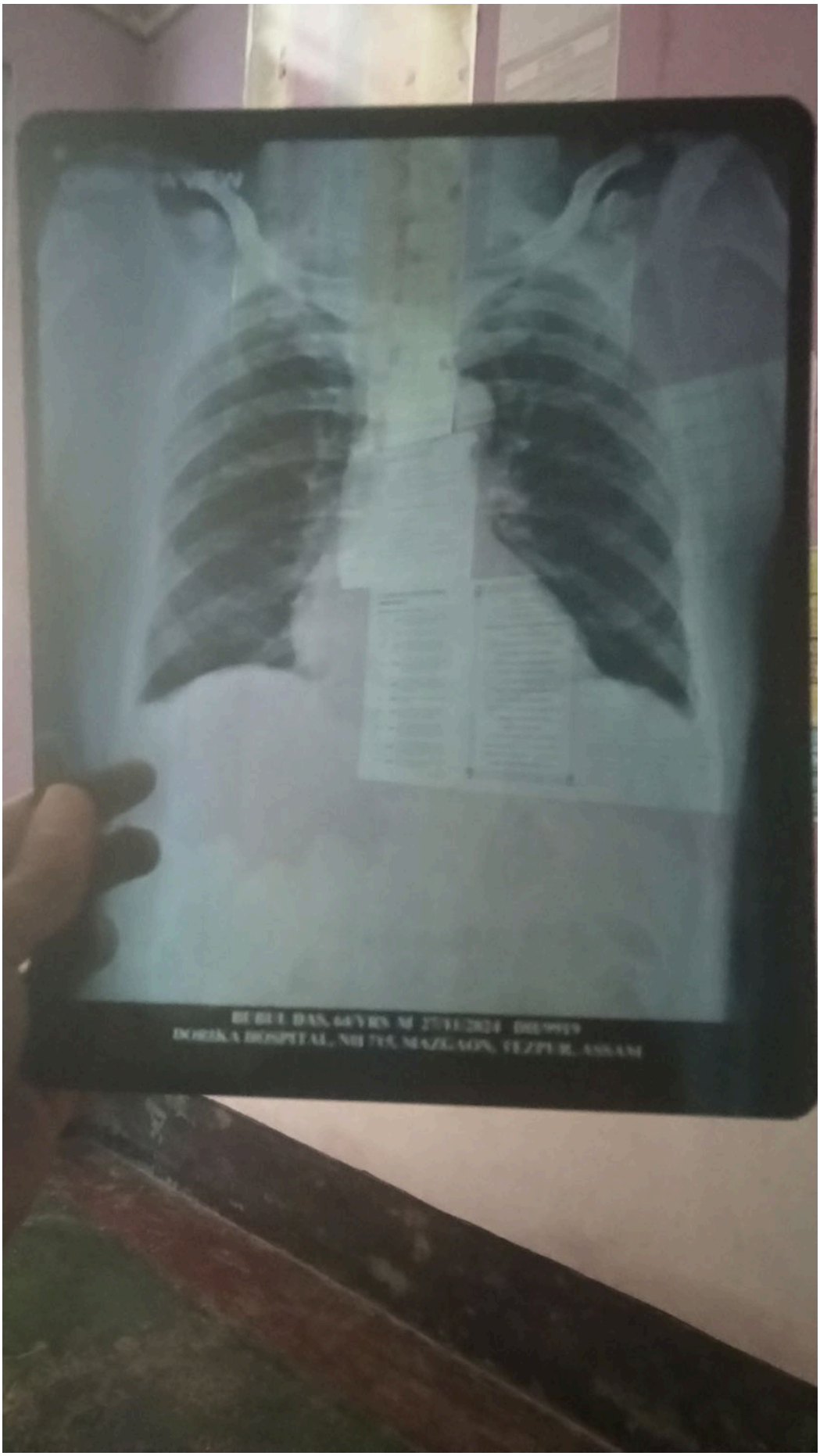
In case of any typing error please get it rectified immediately

24. 27/11/2024 — Chest X-ray film (photograph)

Dorika Hospital, NH 715, Mazgaon, Tezpur, Assam

Photograph of the chest X-ray film held up to light. Film label: "BUBUL DAS, 64/YRS M 27/11/2024 DH/9919 — DORIKA HOSPITAL, NH 715, MAZGAON, TEZPUR, ASSAM". No other text. The written report is on the previous page.

24. 2024-11-27 — Chest X-ray film (photograph) · file: 2024-11-27_Chest-Xray-Film_Dorika-Hospital.jpg



25. 11/03/2026 — Thyroid profile (T3, T4, TSH)

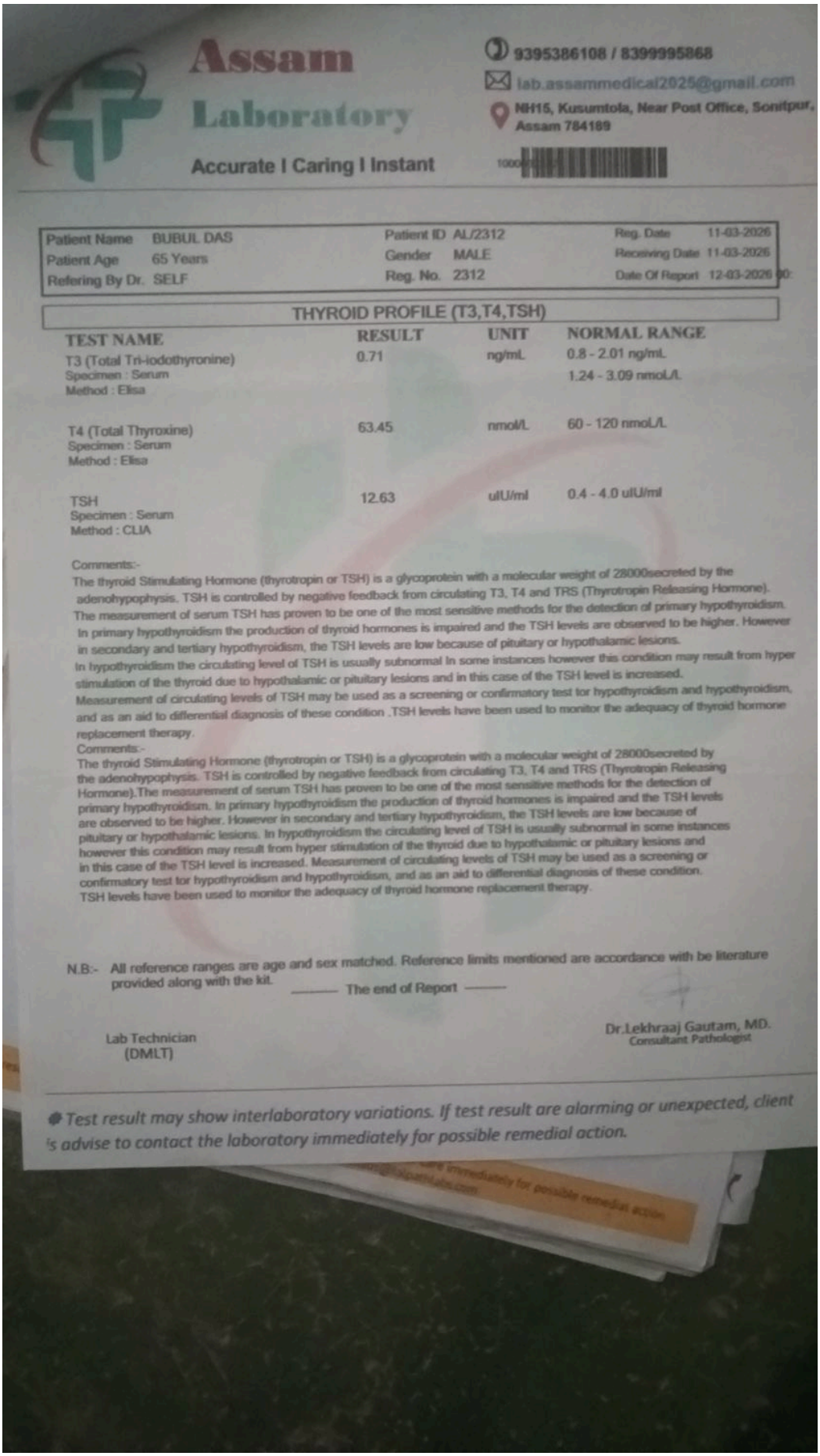
Assam Laboratory ("Accurate | Caring | Instant"), NH-15, Kusumtola, Near Post Office, Sonitpur, Assam 784189 — Dr. Lekhraaj Gautam, MD, Consultant Pathologist

Patient: BUBUL DAS · **Age:** 65 Years · **Gender:** MALE · **Patient ID:** AL/2312 · **Reg. No.:** 2312 · **Referring Dr.:** SELF
Reg. date: 11-03-2026 · **Receiving date:** 11-03-2026 · **Date of report:** 12-03-2026

Test	Result	Unit	Normal range
T3 (Total Tri-iodothyronine) — Elisa	0.71	ng/mL	0.8–2.01
T4 (Total Thyroxine) — Elisa	63.45	nmol/L	60–120 nmol/L
TSH — CLIA	12.63	µIU/ml	0.4–4.0 µIU/ml

Interpretive comments on report: TSH is controlled by negative feedback from circulating T3, T4 and TRH; in primary hypothyroidism TSH levels are elevated, and TSH is a sensitive method for detecting primary hypothyroidism; TSH levels have been used to monitor adequacy of thyroid hormone replacement therapy. N.B.: All reference ranges are age and sex matched. Test results may show inter-laboratory variations. If test results are alarming or unexpected, client is advised to contact the laboratory immediately.

25. 2026-03-11 — Thyroid profile (T3, T4, TSH) · file: 2026-03-11_Thyroid-Profile_Assam-Laboratory.jpg



Assam Laboratory

Accurate I Caring I Instant

9395386108 / 8399995868
lab.assammedical2025@gmail.com
NH15, Kusumtola, Near Post Office, Sonitpur, Assam 784189

Patient Name	BUBUL DAS	Patient ID	AL/2312	Reg. Date	11-03-2026
Patient Age	65 Years	Gender	MALE	Receiving Date	11-03-2026
Refering By Dr.	SELF	Reg. No.	2312	Date Of Report	12-03-2026

THYROID PROFILE (T3,T4,TSH)

TEST NAME	RESULT	UNIT	NORMAL RANGE
T3 (Total Tri-iodothyronine) Specimen : Serum Method : Elisa	0.71	ng/mL	0.8 - 2.01 ng/mL 1.24 - 3.09 nmoL/L
T4 (Total Thyroxine) Specimen : Serum Method : Elisa	63.45	nmol/L	60 - 120 nmoL/L
TSH Specimen : Serum Method : CLIA	12.63	ulU/ml	0.4 - 4.0 ulU/ml

Comments:-
The thyroid Stimulating Hormone (thyrotropin or TSH) is a glycoprotein with a molecular weight of 28000secreted by the adenohypophysis. TSH is controlled by negative feedback from circulating T3, T4 and TRS (Thyrotropin Releasing Hormone). The measurement of serum TSH has proven to be one of the most sensitive methods for the detection of primary hypothyroidism. In primary hypothyroidism the production of thyroid hormones is impaired and the TSH levels are observed to be higher. However in secondary and tertiary hypothyroidism, the TSH levels are low because of pituitary or hypothalamic lesions. In hypothyroidism the circulating level of TSH is usually subnormal In some instances however this condition may result from hyper stimulation of the thyroid due to hypothalamic or pituitary lesions and in this case of the TSH level is increased. Measurement of circulating levels of TSH may be used as a screening or confirmatory test tor hypothyroidism and hypothyroidism, and as an aid to differential diagnosis of these condition .TSH levels have been used to monitor the adequacy of thyroid hormone replacement therapy.

Comments:-
The thyroid Stimulating Hormone (thyrotropin or TSH) is a glycoprotein with a molecular weight of 28000secreted by the adenohypophysis. TSH is controlled by negative feedback from circulating T3, T4 and TRS (Thyrotropin Releasing Hormone).The measurement of serum TSH has proven to be one of the most sensitive methods for the detection of primary hypothyroidism. In primary hypothyroidism the production of thyroid hormones is impaired and the TSH levels are observed to be higher. However in secondary and tertiary hypothyroidism, the TSH levels are low because of pituitary or hypothalamic lesions. In hypothyroidism the circulating level of TSH is usually subnormal in some instances however this condition may result from hyper stimulation of the thyroid due to hypothalamic or pituitary lesions and in this case of the TSH level is increased. Measurement of circulating levels of TSH may be used as a screening or confirmatory test tor hypothyroidism and hypothyroidism, and as an aid to differential diagnosis of these condition. TSH levels have been used to monitor the adequacy of thyroid hormone replacement therapy.

N.B:- All reference ranges are age and sex matched. Reference limits mentioned are accordance with be literature provided along with the kit.

——— The end of Report ———

Lab Technician
(DMLT)

Dr.Lekhraaj Gautam, MD.
Consultant Pathologist

Test result may show interlaboratory variations. If test result are alarming or unexpected, client is advise to contact the laboratory immediately for possible remedial action.

26. 12/03/2026 — OPD slip with prescription (handwritten)

HANDWRITTEN

Rangachakua State Dispensary (ৰঙাচকুৱা ৰাজ্যিক চিকিৎসালয়), Rangachakua, Sonitpur (Assam) — Pin 784182 — Parichalana Samiti, National Health Mission

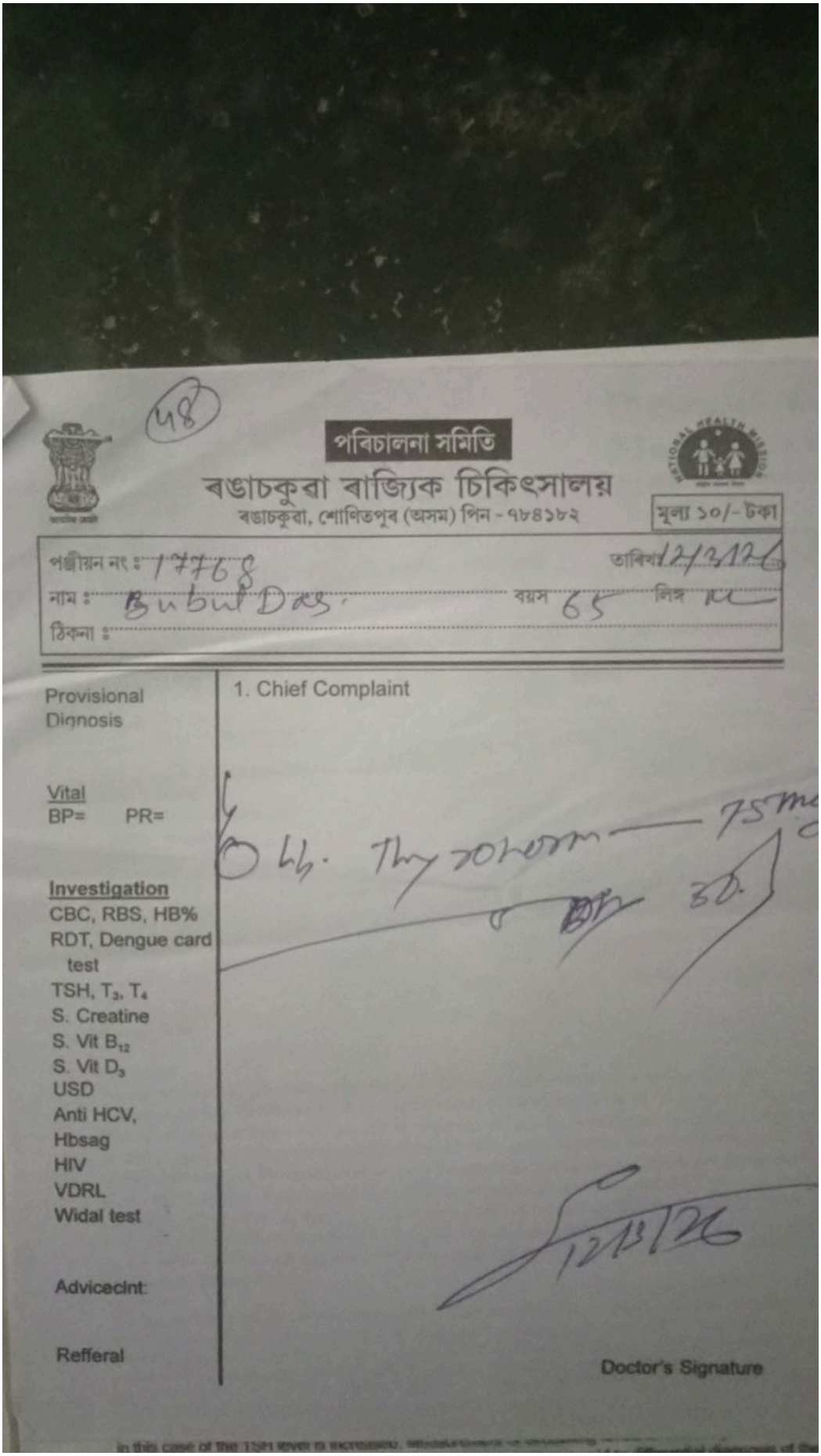
Slip no.: (48) circled · **Registration No.:** 17768 · **Name:** Bubul Das · **Age:** 65 · **Sex:** M · **Date:** 12/3/26 (also signed 12/3/26)

Printed template: Chief Complaint; Provisional Diagnosis; Vital BP=, PR=; Investigation list (CBC, RBS, Hb%, RDT, Dengue card test, TSH T3 T4, S. Creatinine, S. Vit B12, S. Vit D3, USD, Anti HCV, HBsAg, HIV, VDRL, Widal test); Advice; Referral; Doctor's Signature. Vitals and investigation list are not filled / ticked.

Rx (handwritten): Tab. Thyronorm 75 mcg … [illegible] … 30 [?]

> Handwriting — only "Tab. Thyronorm 75 mc[g]" is clearly legible; the remaining dosing text is unclear.

26. 2026-03-12 — OPD slip with prescription (handwritten) · file: 2026-03-12_OPD-Slip_Rangachakua-State-Dispensary.jpg



পৰিচালনা সমিতি

ৰঙাচকুৰা ৰাজ্যিক চিকিৎসালয়

ৰঙাচকুৰা, শোণিতপুৰ (অসম) পিন - ৭৮৪১৮২

মূল্য ১০/- টকা

পঞ্জীয়ন নং : 17768 তাৰিখ 12/3/26

নাম : Bubul Das. বয়স 65 লিঙ্গ F

ঠিকনা :

Provisional Diagnosis

Vital
BP= PR=

Investigation
CBC, RBS, HB%
RDT, Dengue card test
TSH, T_3, T_4
S. Creatine
S. Vit B_{12}
S. Vit D_3
USD
Anti HCV,
Hbsag
HIV
VDRL
Widal test

Adviceclnt:

Refferal

1. Chief Complaint

Lt. Thyronorm — 75mc
30.

12/3/26

Doctor's Signature